THE LOOKING GLASS BEAUTY SCHOOL

Liquid and Powder Acrylic Enhancement CPD Course — Learner Manual

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Continuing Professional Development (CPD) activity — not a regulated qualification.

Document control and permitted use

This manual has been prepared for The Looking Glass Beauty School's Liquid and Powder Acrylic Enhancement CPD Course. It is designed to support qualified beauty or nail professionals in updating and demonstrating safe acrylic enhancement knowledge and practical competence. It does not replace product-specific training, an insurer's requirements, local-authority conditions, a Safety Data Sheet (SDS), or medical advice.

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> ***SCOPE BOUNDARY — This course teaches liquid-and-powder acrylic overlays and tip or form extensions on healthy natural fingernails. Gel polish, builder gel and hard gel are compared so learners can recognise the systems and work within scope; this CPD course alone does not qualify a learner to provide those separate services. Electric-file work on the natural nail is not taught or assessed here.***

Course author

I am Jessica Jane Preston, a qualified nail professional, beauty educator and salon co-founder with more than fourteen years of industry experience. I hold a VTCT Level 2 NVQ Diploma in Nail Services and an Award in Education and Training. Since 2021 I have delivered classroom-based

beauty education and have trained more than 400 learners. My CV and qualification certificates are available from the school on request.

Important professional boundaries

- A nail professional observes, records, adapts, postpones and refers; a nail professional does not diagnose a disease or prescribe medical treatment.
- Never apply uncured product to skin as a 'patch test'. Suspected acrylate allergy requires medical assessment and, where appropriate, formal patch testing by a suitably qualified clinician.
- Do not rely on this manual to override a product label, SDS, manufacturer system guide, local licence condition or insurer instruction.
- Where an urgent reaction includes breathing difficulty, collapse, rapidly increasing facial or throat swelling, or other signs of a medical emergency, call 999. For non-emergency concerns use a pharmacist, GP or NHS 111 as appropriate.

Chapter 1 — Course specification and learning journey

What learners can expect, who the course is for, and how competence is assessed

Course title

Liquid and Powder Acrylic Enhancement CPD Course

Course description

The Liquid and Powder Acrylic Enhancement CPD Course is a practical skills-development activity for existing beauty and nail professionals. Learners revisit nail anatomy and the conditions that require a service to be adapted, postponed or refused; explain the chemistry of monomer and polymer, the mix ratio and free-radical polymerisation; and build a balanced enhancement with a correctly placed apex. Through tutor demonstration and supervised practice, learners prepare the natural nail, fit tips or sculpting forms, apply acrylic in zones with controlled bead consistency, refine the structure, and carry out infills and safe removal. The course also covers dust and vapour control, allergy prevention, equipment care, UK legal responsibilities and honest advertising.

Successful completion requires a theory assessment, a directly observed practical assessment and three satisfactory post-course case studies. This activity is Continuing Professional Development and does not replace or confer a regulated Level 3 nail qualification.

> ***SCOPE BOUNDARY — This course teaches liquid-and-powder acrylic overlays and tip or form extensions on healthy natural fingernails. It does not cover toes, electric-file work on the natural nail, or the correction of significant nail-plate damage or disease.***

Entry requirements and prerequisites

- Learners must be aged 18 or over unless the school's insurer has confirmed an alternative minimum age in writing.
- Learners must hold at least a Level 2 Beauty Therapy, Nail Services or equivalent qualification including manicure knowledge, nail structure and safe salon practice. A completed gel-polish or equivalent CPD course alone is not sufficient preparation for an enhancement system.
- Learners must be able to read and follow English-language product labels, Safety Data Sheet information, consultation documents and assessment instructions. Reasonable adjustments will be considered without removing competence or safety requirements.
- Learners must disclose any access, health or allergy needs relevant to safe participation, and must provide a suitable adult model if required by the booking instructions. Learners with a known acrylate sensitivity must not undertake this course.
- Before offering the treatment commercially, learners must confirm that their qualification route, this CPD training and intended working arrangements are accepted by their own insurer and local authority.

Duration and delivery structure

Component	Time	Evidence
Pre-course reading and knowledge activity	3 hours	Completed learner workbook prompts
Tutor-led classroom and practical day	7.5 guided learning hours, excluding breaks	Attendance, questioning and direct observation
Three post-course case studies	Approximately 9 hours total	Consultation, treatment record, photographs with consent, reflection and educator feedback
Total learner effort	Approximately 19.5 hours	All evidence retained in the learner portfolio

The lesson plan for the taught day (document A05 in the Tutor and Assessor Pack) sets out the timetable and matches these hours: 7.5 guided learning hours on the day, 3 hours of pre-course reading and about 9 hours of case-study work, 19.5 hours in all. Learners bring a suitable adult model for the afternoon practical and the assessment; the model is confirmed at booking.

Learning outcomes

- LO1 — Explain the purpose, benefits, limitations and client suitability of liquid-and-powder acrylic, and compare it with builder gel, hard gel and gel polish.
- LO2 — Identify the structures of the nail unit and recognise conditions that require an enhancement service to be adapted, postponed or refused.
- LO3 — Conduct and record a professional consultation, obtain informed consent and create an individual treatment plan for an enhancement client.

- LO4 — Explain monomer, polymer, initiator and catalyst, free-radical polymerisation, the mix ratio, set time and the difference between EMA and MMA products.
- LO5 — Select, handle, store and dispose of acrylic products and equipment in accordance with risk assessment, COSHH, manufacturer instructions and current UK requirements.
- LO6 — Control dust and vapour exposure and explain why acrylate overexposure causes sensitisation.
- LO7 — Prepare the natural nail for an enhancement without avoidable damage to the plate or living tissue.
- LO8 — Fit and blend a tip, or fit a sculpting form, achieving a sidewall-to-sidewall fit with no gaps.
- LO9 — Apply acrylic in zones with controlled bead consistency to build a balanced structure with a correctly placed apex, straight sidewalls and an even C-curve.
- LO10 — Refine, finish, infill, repair and safely remove an acrylic enhancement, and provide appropriate aftercare.

Assessment strategy and pass requirements

Theory: a written paper of 38 questions totalling 50 marks, with partial credit for multi-part questions. The pass mark is 80% (40/50). Any answer that instructs a learner to use MMA or an unlabelled product, apply product onto living tissue, work with a wet bead, file without extraction, prise or drill an enhancement off, file the natural nail with an electric file without separate training, or conceal an active infection must be corrected before a pass is awarded.

Practical: direct observation on a suitable adult model, who the learner brings to the taught day. The learner must achieve at least 80% overall (32 of 40 on the marking sheet, document A04) and pass every safety-critical criterion. Cosmetic finish cannot compensate for unsafe preparation, product control, skin contact, dust or vapour control, or removal.

Where the assessment papers are: the theory paper (document A02), its mark scheme (A03) and the practical observation marking sheet (A04) are held in the Tutor and Assessor Pack and are not printed in this manual. The practical is marked on twenty criteria; the seven below are safety-critical and every one must be met for a pass, whatever the overall score:

- No wet product, primer or monomer on skin at any point; a hairline gap kept at the proximal and lateral folds and at the hyponychium.
- A correct mix ratio throughout: no wet beads placed, no bead left to flood the folds, product on skin removed with the brush before it sets.
- Dust and vapour controlled: source-capture extraction on for every filing and removal, the dappen dish lidded between beads, bottles capped, dust damp-wiped.
- No cutting of living tissue; no filing into the lateral folds; the natural nail de-shined with 240 grit or finer, not thinned; no electric file on the natural nail.
- The service refused or adapted where a contraindication is present; suspected infection or reaction never covered; no fresh product applied over lifted product.

- Removal and infill by the methods in Chapter 15: bulk reduced by hand file and product soaked, never prised, nipped, drilled or filed down to the plate.
- Hygiene: single-use items not reused, tools decontaminated between clients, the blood or skin-break procedure followed if needed.

Case studies: three complete acrylic services after the taught day, each with consultation, before and after images where separate consent is held, product record, treatment notes, aftercare and reflection. All three must be satisfactory.

Certification: the CPD certificate is released only after theory, practical and case-study requirements are complete. Attendance alone is not sufficient.

Chapter 2 — Liquid-and-powder acrylic: purpose, benefits and professional scope

Understanding the system before learning the technique

What liquid-and-powder acrylic is

A liquid-and-powder acrylic enhancement is formed by combining a liquid monomer with a polymer powder. The two react chemically and harden without a lamp. The professional draws a bead onto the brush, places it on the nail and shapes it while it is workable; the bead then sets, is refined by filing, and is finished to a high shine or coated with colour.

Because the material sets by chemical reaction rather than by light, there is no cure time to control with a lamp. Instead the professional controls the result through the mix ratio, bead placement and working speed.

Benefits and limitations

Potential benefit	Professional limitation
Strong, durable structure suitable for clients who are hard on their nails	Strength comes from correct architecture, not from thickness. A thick, badly balanced enhancement is weaker than a thin, well-balanced one.
Can add length using tips or sculpting forms	Added length increases leverage on the natural nail. Length must be agreed against the client's work and lifestyle, not simply requested.
Sets without a lamp, so no cure-related under-curing	Set time varies with product, room temperature and ratio. A bead that sets too fast cannot be shaped; too slow and it slumps into the cuticle area.
Colour and design can be embedded within the product	Coloured powders vary in density and may change the working time.
Maintained by infill rather than full removal	Infills must rebalance the structure, not simply fill the regrowth gap. Repeated filling

	without rebalancing moves the apex and causes breakage.
Removed by soaking off in acetone	Removal is slower than gel polish. Forced prising or nipping removes layers of the natural nail plate and is never acceptable.

Who the treatment may suit

- A client with healthy natural fingernails and intact surrounding skin who wants added strength, length or shape.
- A client who understands that enhancements are a commitment requiring infills every two to three weeks and professional removal.
- A client whose chosen length and shape are practical for their work, caring responsibilities, sport and hobbies.
- A client whose consultation reveals no active contraindication and whose treatment can be completed within the learner's competence, insurer conditions and product instructions.

When another service may be more appropriate

- Gel polish suits a client who wants colour on a healthy natural nail without added structure.
- A builder-gel overlay may suit a client who wants modest reinforcement with a softer feel and a simpler removal, where the professional is separately trained in that system.
- No enhancement should be used to conceal a suspected infection, unexplained nail change or active reaction.
- A client who repeatedly picks or prises product off may damage the natural nail more than the enhancement protects it. Address the behaviour before continuing the service.

Comparing the principal nail systems

System	How it hardens	Removal	Training boundary
Liquid-and-powder acrylic	Chemical reaction between monomer and polymer; no lamp	Soak off in acetone, usually after reducing bulk	Practical focus of this course
Builder gel in a bottle	Light cure in a compatible lamp	Product-specific: soak off, file off or combined	Separate training required
Hard gel	Light cure; rigid or semi-rigid	File off; acetone does not normally remove it	Separate training required
Gel polish	Light cure; thin colour coating with no structure	Soak off as directed	Separate training required
Traditional lacquer	Air dries by solvent evaporation	Lacquer remover	Manicure competence still required

QUICK CHECK — A client asks for long stiletto nails for a job that involves lifting stock. Explain the leverage problem in plain terms, offer a shorter and stronger shape, and record what was agreed. Meeting a request that will break within days is not good service.

Where liquid-and-powder acrylic came from

The first acrylic nail was an accident. In the mid-1950s a Philadelphia dentist, Frederick Slack, repaired a nail he had damaged at work with dental acrylic and a piece of foil; the result held, and with his brother he patented a nail system and the trade in 'porcelain' nails began. Those early liquids were methyl methacrylate (MMA), borrowed straight from dentistry. Through the 1960s and 1970s complaints of allergy, nail-plate damage and loss of sensation in the fingertips mounted, and by the end of the 1970s the United States regulator had acted against MMA nail products and told manufacturers that their use in nail enhancements was inappropriate. The professional industry moved to ethyl methacrylate (EMA), the monomer in every reputable system today. The service in this manual is that EMA system: a more flexible product that bonds to a properly prepared plate without the extreme hardness that made MMA enhancements shatter and take nail plate with them. Light-cured gels arrived in the 1980s as a lower-odour alternative; liquid-and-powder acrylic remains the strongest and most sculptable of the systems and the one most professionals learn for length.

Service time, pricing and promotion

A full set on tips or forms commonly takes two to two and a half hours for a newly trained professional and 75 to 100 minutes with experience; an overlay on the natural nail 60 to 90 minutes; an infill and rebalance 60 to 75 minutes; a removal 45 to 60 minutes including the 20 to 30 minute soak. French, coloured powders or an embedded design add 15 to 30 minutes. Price the service on your time, product cost and your market, not on what a competitor charges. The points below are a starting framework; set your own figures.

- Full set: the highest price on the nail menu, reflecting time, skill and product; sculpted forms are often priced above tips.
- Infill and rebalance: priced as a service in its own right, not a discount; it takes most of the time of a full set and all of the skill.
- Removal: a separate charge, reduced or waived when the client rebooks a new set the same day.
- Repair of a single nail within the first few days: at your discretion, and only once the cause has been looked at.
- French, colour or embedded design: a supplement on top of the set price.

Promotion that works: photograph every set from above, from the side and from the barrel, with consent and in the same light; a side view showing a clean apex and a barrel view showing

an even C-curve are the pictures other professionals notice. Describe the service honestly in line with Chapter 10 (strong and long-lasting, never 'guaranteed not to lift' or 'does no damage'). Book the infill at the end of every appointment, two to three weeks ahead: it protects the natural nail and keeps the diary full. Tell clients plainly who acrylic is for and who it is not for; the client who leaves with the right service is the one who comes back.

Chapter 3 — Nail anatomy, physiology and growth

The structures a nail professional must protect

The nail unit is more than the visible plate. It includes living skin, growth tissue, protective folds and supporting structures. Understanding the difference between living and non-living tissue is essential because a cosmetic service should not deliberately cut, abrade or seal over damaged living tissue.

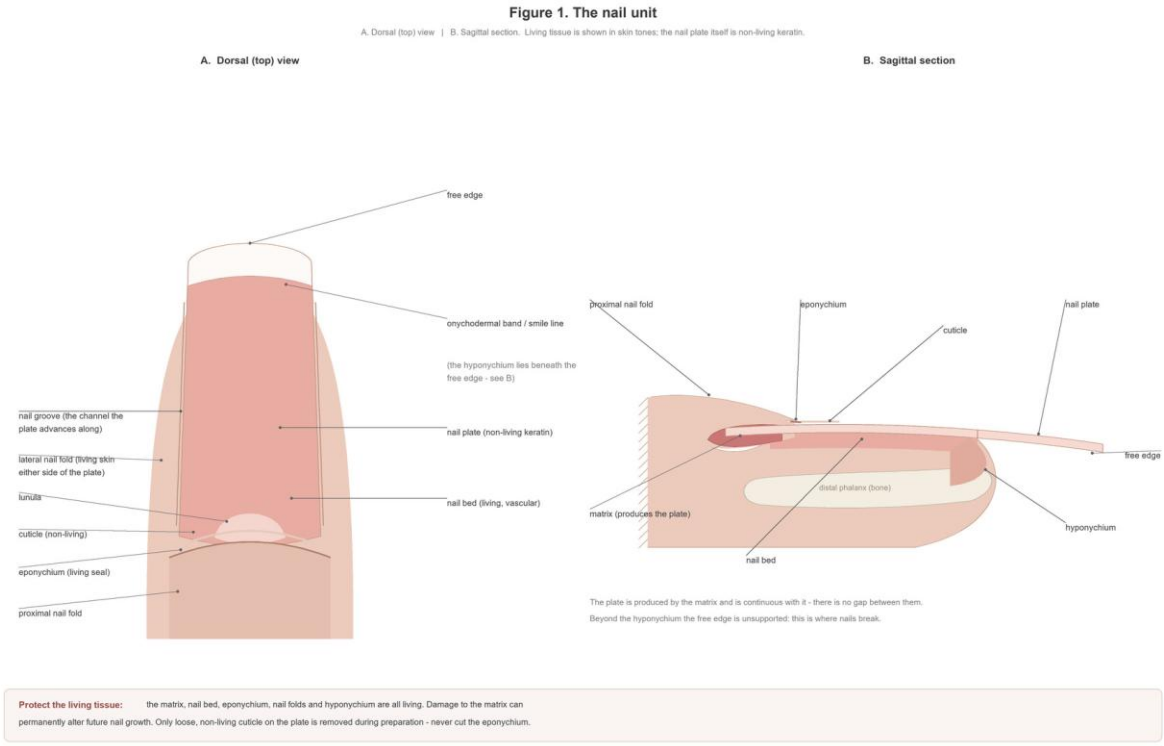


Figure 1. The nail unit. A: dorsal view. B: sagittal section. The matrix produces the nail plate and is continuous with it. The free edge beyond the hyponychium is unsupported.

Original illustration © The Looking Glass Beauty School, 2026.

Structures and functions

Structure	Function and professional relevance
Matrix	Living growth tissue beneath the proximal nail fold. Matrix cells divide, flatten and keratinise to form most of the nail plate.

	Significant injury can alter future nail growth; the matrix must never be deliberately filed, cut or probed.
Nail plate	The visible, hard but flexible keratin structure. It contains no blood vessels or nerves. Its layers can be thinned or delaminated by over-filing, peeling product, trauma and repeated dehydration.
Nail bed	Living, vascular tissue beneath the plate that supports and anchors it as it advances. It contributes to the pink appearance of a healthy attached nail. Separation of plate from bed is onycholysis.
Proximal nail fold / proximal nail wall	Living skin at the base of the nail that protects the emerging plate and matrix. It is not 'dead cuticle' and should not be aggressively pushed back or cut.
Lateral nail folds / nail walls	Living skin along each side of the plate. They protect the nail unit and guide the plate. Filing into these folds can cause pain, breaks in the barrier and infection risk.
Cuticle	Thin non-living tissue shed from the underside of the proximal nail fold and carried onto the plate. Only loose or adhered non-living tissue is gently removed during preparation.
Eponychium	Living tissue associated with the proximal nail fold and protective seal. It is often incorrectly called the cuticle. Do not cut it simply to create a visually 'clean' margin.
Lunula	The visible pale crescent of the distal matrix on some nails, most obvious on thumbs. Its visibility varies and absence from view is not automatically abnormal.
Hyponychium	Living tissue beneath the free edge where the nail bed ends. It forms a protective barrier. Do not dig under, cut or separate it while cleaning or shortening the nail.
Free edge	The part of the plate extending beyond the fingertip. It can be shortened and shaped while the finger and sidewalls are supported.
Nail grooves	Channels at the sides in which the plate advances. Product should not be flooded into these areas.
Onychodermal band	The transition between nail bed and hyponychium, sometimes visible as a narrow colour change beneath the plate. It is part of

	the distal protective seal.

How the nail grows

New nail-plate cells are produced in the matrix. As cells mature they flatten, lose their nuclei, fill with keratin and are pushed distally over the nail bed. A fingernail commonly grows at around 3 mm per month, although the rate varies between people, fingers and seasons. A completely new fingernail may take roughly four to six months to grow from matrix to free edge. Growth may be slower with age, illness, impaired circulation, nutritional deficiency, some medication and significant trauma. Use a range and explain variation rather than promising an exact regrowth date.

Factors that can influence nail appearance and growth

- **Local trauma:** crushing, repeated tapping, biting, picking, over-filing and forced product removal can affect the plate, bed or matrix.
- **Water and chemicals:** repeated wetting/drying, detergents and solvents can contribute to brittleness and delamination.
- **General health:** illness, fever, inflammatory skin disease, anaemia and other conditions may be associated with nail changes, but a nail professional must not diagnose the cause.
- **Medication and medical treatment:** some medicines and treatments can change growth, colour, thickness or fragility. Record only information relevant to treatment safety and refer when the change is new or unexplained.
- **Age and individual variation:** ridging, thickness and growth rate vary. A difference is not automatically disease, but a sudden or marked change should not be concealed.
- **Product and technique: incorrect preparation, a wet mix ratio, skin contact, thick or badly balanced product, aggressive removal and excessive enhancement length can damage or destabilise the service.**

Protecting the natural nail

- Use the least abrasive preparation permitted by the product manufacturer. Do not 'etch' grooves into the plate.
- Support the finger and use light, controlled strokes. Stop if the client reports heat, pain or tenderness.
- Keep wet product, primer and monomer away from all skin and from the underside of the free edge unless the system specifically directs otherwise.
- Never use the enhancement as a tool to prise, scrape or pick. Preventing mechanical trauma is part of aftercare.
- If the nail plate is visibly thin, sore or over-filed, postpone the enhancement and allow recovery rather than hiding the damage.

Chapter 4 — Nail diseases, disorders and recognition

Observe, record, decide and refer — never diagnose

A disease is a pathological process such as infection or inflammatory illness. A disorder or nail change may be traumatic, congenital, behavioural or associated with general health. The categories overlap, and appearance alone is not enough to diagnose. A nail professional should describe what is seen, ask relevant questions, decide whether treatment is safe and refer when uncertain.

> ***FOUR-STEP DECISION — OBSERVE the nail and surrounding skin. ASK when it began, whether it hurts or itches, and whether it is changing. DECIDE whether to proceed, adapt or postpone within your competence and insurance. RECORD the decision and REFER when there is infection, inflammation, unexplained change, pain, broken skin or uncertainty.***

Core diseases and disorders

Condition or change	What may be observed	Professional action
Onychomycosis (fungal nail infection)	Thickened, brittle, crumbly or discoloured nail; debris; lifting may occur. Psoriasis and trauma can look similar, so visual inspection is not a diagnosis.	Do not apply or reapply an enhancement. Avoid cross-contamination, use single-use items appropriately and advise the client to seek pharmacist, GP or podiatry advice. Treatment is medical, not a salon service.
Onycholysis	Separation of the nail plate from the bed, often beginning distally. The detached area appears white, grey or discoloured. Causes include trauma, psoriasis, fungal infection, irritants and other conditions.	Do not cover active, extensive, painful, discoloured or unexplained separation. Keep the area dry, avoid probing under the plate and refer. A stable, diagnosed residual change may be treated only with insurer approval and no contraindication.
Onychophagy (nail biting)	Short, irregular free edge; surrounding skin may be picked, sore or broken. Severity varies.	Do not treat over broken, bleeding, inflamed or infected skin. Mild biting with intact skin may allow a short, carefully built overlay or extension once the plate has enough length to hold a tip or form; expectations and maintenance must be realistic, and the client must

		agree not to bite the product.
Nail psoriasis	Pitting, thickening, colour change, crumbling, onycholysis or oil-drop/salmon patches may occur. It is inflammatory and not contagious.	Do not diagnose. Avoid treatment during an active painful flare, where skin is broken, or where the presentation is uncertain. Stable diagnosed psoriasis may sometimes be treated with medical/insurer guidance and gentle technique.
Paronychia	Red, swollen, warm and tender nail fold; pus may be present. It may be acute bacterial infection or chronic inflammation associated with repeated wet work.	Refuse treatment. Do not drain, cut, massage or cover the area. Advise prompt pharmacy/medical assessment, especially if pain, pus, spreading redness or systemic illness is present.
True pterygium	Scar-like adhesion or abnormal forward growth of tissue involving the nail unit. Dorsal pterygium can split or narrow the plate; pterygium inversum affects tissue beneath the free edge.	Do not confuse true pterygium with ordinary cuticle. Do not cut or push adherent living tissue. Refer for medical assessment before any enhancement.
Onychogryphosis (thick dystrophic nail)	Marked thickening, curvature and distortion of the nail, which may trap debris or be difficult to cut. It usually affects toenails and is often associated with pressure, age or neglect.	An acrylic enhancement is inappropriate. Do not aggressively file or coat. Refer to a podiatrist/medical professional, especially where diabetes, poor circulation, pain or infection risk is present; do not attempt aggressive reduction unless separately qualified and insured.
Brittle or splitting nails (onychoschizia/onychorrhexis)	Peeling layers, transverse splitting or longitudinal ridges/splits. Repeated water/chemical exposure and trauma can contribute; systemic causes are also possible.	Mild stable brittleness with intact skin may receive a carefully built, short enhancement if suitable. New, severe, painful or multiple-nail changes require referral. Do not promise that an enhancement will cure the cause.
Koilonychia (spoon nails)	A concave or spoon-shaped plate. It can be congenital,	If new or unexplained, refer. Do not use product to

	traumatic or associated with health conditions such as iron deficiency.	disguise a possible health sign. If longstanding, medically reviewed and intact, treatment may be adapted within insurer conditions.
Duckbill / flared nail shape	The free edge widens or fans outward. This may be the natural growth pattern or result from previous enhancement architecture.	Not automatically a disease. Keep length practical, refine only without weakening sidewalls, and choose a flattering shape. A well-placed apex and a form fitted to the natural C-curve can improve the look; do not pinch the sidewalls to correct it.
Leukonychia (white spots)	Small opaque white marks commonly follow minor matrix trauma and grow out with the plate. Widespread, powdery or changing white areas can have other causes.	A few stable trauma-related spots on an otherwise healthy nail can usually be coated. Do not diagnose. Refer powdery, spreading, lifting, painful or unexplained changes.
Beau's lines	Transverse depressions caused by temporary interruption of nail growth. They may follow illness, injury or other stress.	If shallow, growing out and the nail is healthy, a gentle cosmetic service may be possible. New, deep, multiple or unexplained lines should be referred.
Splinter haemorrhage	Fine red-brown or black lines beneath the plate, often from minor trauma but sometimes associated with health conditions.	If clearly linked to minor trauma and growing out, record and monitor. Multiple, recurrent or unexplained marks require medical advice. Never cover a suspicious dark streak without referral.
Subungual melanoma warning sign	A new or changing dark longitudinal band, pigment spreading onto surrounding skin, nail destruction, bleeding or a persistent unexplained lesion.	Do not cover. Urgently advise GP assessment. Nail melanoma is uncommon, but delay caused by cosmetic concealment is unacceptable.

Recognising what you are looking at

The photographs below show presentations you may meet at the consultation. Use them to recognise and describe what you see — never to diagnose. Where a nail does not look like any of these, that is itself a reason to pause and refer.



Figure 2. Onychomycosis affecting two fingernails. Yellow-brown discolouration, thickening, crumbling of the plate and separation at the free edge. Do not apply or reapply an enhancement — treatment is medical, not a salon service.

Photograph © Grook Da Oger, via Wikimedia Commons, CC BY-SA 3.0.



Figure 3. Onychomycosis more commonly affects toenails, as here: marked thickening, yellow-white crumbling and debris beneath the plate. Psoriasis and repeated trauma can look similar — visual inspection is never a diagnosis.

Photograph © James Heilman, MD, via Wikimedia Commons, CC BY-SA 3.0.



Figure 4. Onycholysis of the ring and little fingernails. The plate has separated from the bed beginning at the free edge, and the detached area appears pale and discoloured. Do not cover active, extensive or unexplained separation.

Photograph © CopperKettle, via Wikimedia Commons, CC BY-SA 3.0.

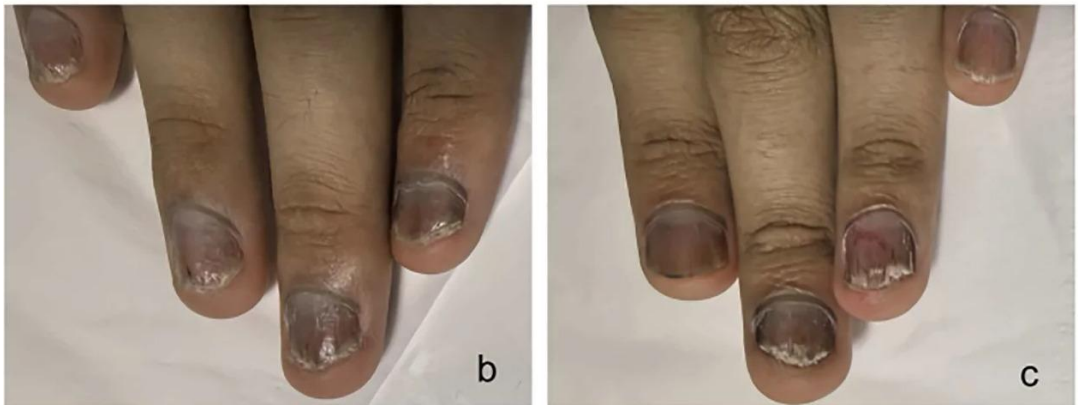


Figure 5. Nail psoriasis. Crumbling of the plate, subungual debris, colour change and separation at the free edge. Psoriasis is inflammatory and not contagious, but it can look very like fungal infection — which is exactly why appearance alone is never a diagnosis. Avoid treatment during an active flare, where skin is broken, or where the presentation is uncertain.

Photograph © Huang Y & Wen S, Frontiers in Immunology (2026), doi:10.3389/fimmu.2026.1941127, CC BY 4.0, cropped. Paediatric patient.



Figure 6. Acute paronychia. The nail fold is swollen, red and tender, with pus present. Refuse treatment. Do not drain, cut, massage or cover the area; advise prompt pharmacy or medical assessment.

Photograph © Mohammad2018, via Wikimedia Commons, CC BY-SA 4.0.



Figure 7. Chronic paronychia. The cuticle seal is lost, the proximal nail fold is thickened and bolstered, and the plate shows ridging and colour change. Commonly associated with repeated wet work.

Photograph © Daifallah M. Al Aboud, M.D., via Wikimedia Commons, CC BY 4.0.

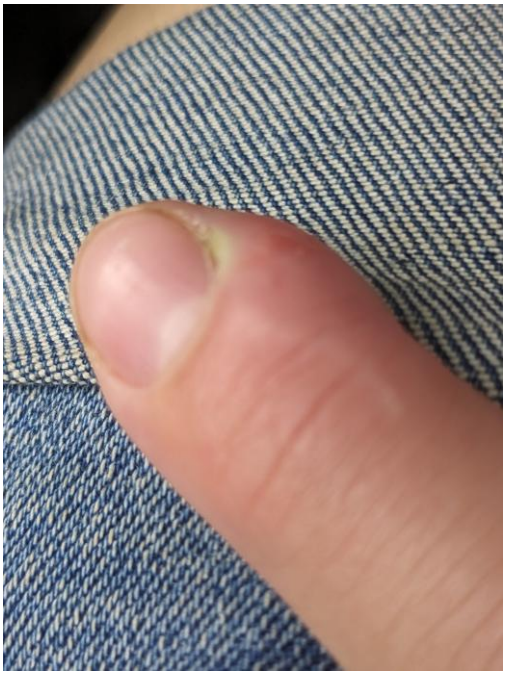


Figure 8. Early paronychia. Localised swelling and a small pus point at the nail fold, with the skin barrier already broken. Even at this stage the service is refused and the client referred.

Photograph © Hannah Clover, via Wikimedia Commons, CC BY 4.0.



Figure 9. Leukonychia punctata. A single opaque white spot following minor matrix trauma, which grows out with the plate. A few stable spots on an otherwise healthy nail can usually be coated — but do not diagnose.

Photograph © BrotherLongLegs, via Wikimedia Commons, CC BY-SA 4.0.



Figure 10. Longitudinal melanonychia — a dark band running the length of the plate. DO NOT COVER. A new or changing band, pigment spreading onto the surrounding skin, nail destruction or bleeding requires urgent GP assessment.

Photograph © Seungjun Lee & Seokchan Eun, via Wikimedia Commons, CC BY 4.0.

Green nail and suspected bacterial infection

Green, green-black or yellow-green discolouration beneath a lifted enhancement is commonly associated with colonisation by Pseudomonas bacteria in a moist space, although colour alone cannot confirm the organism. It is not correctly described as 'green fungus'. The professional response is not to hide the stain or prescribe a home chemical remedy.

1. Stop the service and do not apply fresh product over the discolouration.
2. If an existing enhancement can be removed without cutting skin or over-filing, remove it using the correct product method and within competence. If safe removal is uncertain, refer to a suitably qualified professional.
3. Keep the nail clean and dry. Do not dig under separated plate, buff away colour or use household bleach, neat vinegar, antibiotics or another treatment unless directed by a healthcare professional.
4. Advise the client to seek medical or pharmacy advice, particularly where there is pain, swelling, pus, spreading redness, diabetes, reduced immunity or persistent discolouration.
5. Discard contaminated single-use items, quarantine reusable tools and complete the full cleaning/disinfection or validated sterilisation process required by the risk assessment. Record the product, affected nails, observations, action and referral.

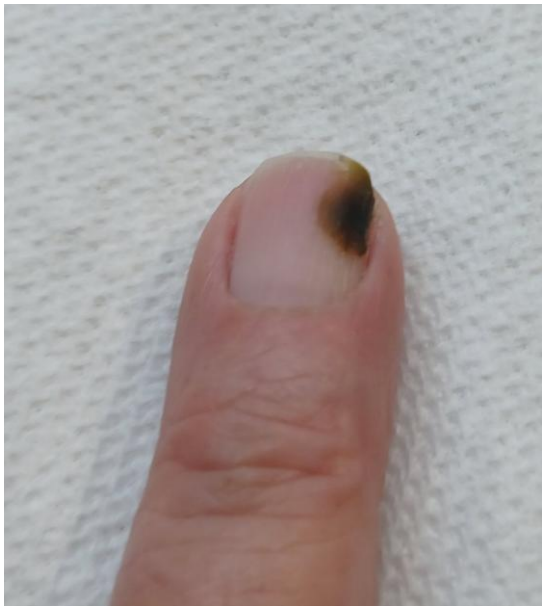


Figure 11. Green nail (chloronychia). Green-black discolouration associated with bacterial colonisation in a moist space. It is not 'green fungus'. Do not apply fresh product over the discolouration.

Photograph by Ekem, via Wikimedia Commons, CC0 1.0 (public domain dedication).



Figure 12. Green nail with associated onycholysis. The discolouration sits beneath a separated area of plate. Do not buff away the colour or use household remedies; keep dry, refer, and decontaminate tools.

Photograph by Ekem, via Wikimedia Commons, CC0 1.0 (public domain dedication).

Contraindications that are not diagnoses

The following findings or circumstances can prevent or restrict an acrylic enhancement service even when no named nail disease has been diagnosed:

- Known or suspected acrylate/methacrylate allergy, or a previous reaction to gel, acrylic, nail glue or adhesive products.
- Active eczema, dermatitis, psoriasis flare or unexplained rash on the hands, fingers or treatment area.
- Cuts, abrasions, bleeding, burns, blisters, swelling, heat, tenderness or recent trauma around the nail.
- A wart or other contagious lesion where the service could touch or spread it.
- Recent surgery, severe nail damage, significant plate thinning or pain on light pressure.
- A new, changing, unexplained or suspicious colour/shape change.
- A client unable to sit still for the length of the service, keep the hands on the desk, follow aftercare, communicate discomfort or give informed consent.
- Any medical condition, treatment or medication for which the insurer or manufacturer requires medical advice, adaptation or refusal. Diabetes and pregnancy are not automatic universal bans; assess individually, avoid injury and follow insurer/product guidance.

> ***DO NOT USE GP LETTERS AS A BLANKET TRANSFER OF RESPONSIBILITY — A doctor's note does not make an unsuitable treatment safe and does not override the professional's competence, product instructions or insurance. Ask only for relevant confirmation, make your own treatment decision and record the rationale.***

Chapter 5 — Consultation, treatment planning and consent

A safe service begins before the product is opened

Purpose of the consultation

A consultation is a structured two-way conversation and assessment. It establishes what the client wants, whether the treatment is appropriate, how it should be adapted and whether the client understands costs, maintenance, limitations and risks. A signed form supports the process but is not a substitute for observation, professional judgement or dialogue.

The consultation sequence

1. Create privacy and explain why relevant questions are being asked. Confirm the client's identity, age and preferred communication needs.
2. Clarify the desired result: colour, finish, shape, length, event, workplace requirements, budget and expected maintenance.
3. Ask about previous nail services, wear, lifting, removal methods, reactions, allergies and whether any nails are painful or changing.
4. Collect only health and medication information that is relevant to treatment safety. Explain that health data is sensitive and how it will be protected.

5. Visually and manually assess the nail plate, free edge, folds, surrounding skin and each hand. Compare nails; do not conceal suspicious findings.
6. Identify contraindications and decide to proceed, adapt, postpone or refuse. Explain the reason respectfully without diagnosing.
7. Agree a realistic treatment plan: product system, overlay or extension on a tip or form, length, shape, colour or finish, service time, price, infill interval, removal and aftercare.
8. Explain foreseeable risks including temporary dryness, a warm sensation as product sets, lifting, breakage, allergy/sensitisation risk and harm from picking or forced removal.
9. Obtain treatment consent. Obtain separate, optional consent for identifiable photographs or marketing; refusal of marketing consent must not prevent treatment.
10. Record products, batch numbers where practical, tip or form used, length and shape, observations, adaptations and aftercare. Update the record at every appointment.

Treatment planning

A treatment plan converts consultation findings into a safe, individual service. It should be specific enough that another competent professional can understand what was agreed and what happened.

Planning area	What to decide and record
Client goal	Overlay or extension, length, shape, colour or finish, occasion, desired wear and the maintenance commitment
Nail condition	Plate thickness, flexibility, free-edge condition, skin integrity, relevant variations and contraindication decision
Lifestyle	Work, handwashing/wet work, chemicals, sport, gardening, caring, musical instruments, habitual picking and time available for maintenance
System	Named monomer, powder, primer and dehydrator from one system; tips or forms; the Safety Data Sheets held
Adaptation	Shorter length, overlay instead of extension, reduced abrasion, one nail excluded, additional removal time, no cuticle nipping, or treatment postponed
Service variation	New full set on tips or forms, overlay on the natural nail, infill and rebalance, repair, removal, or removal and a new set
Risk and consent	Benefits, limitations, allergy information, aftercare, cost and separate photo consent
Review	Expected maintenance/removal date, what the client should report, and outcome at the next visit

Consent is informed and ongoing

- The client must understand what will be done, material risks, alternatives and the right to stop. A signature does not remove the duty of care.
- Check consent again if the plan changes, discomfort occurs or another product is proposed.
- Do not use coercive wording such as 'I accept all responsibility'. A professional remains responsible for acting competently.
- For case-study photographs, obtain model consent for the educational purpose and specify who will see the images, how long they will be kept and whether the face/identity will be excluded.
- If consent cannot be freely and clearly given, do not treat.

Consultation scenario

> ***LEARNER ACTIVITY — A client asks for long almond nails for a warehouse job, reports that previous acrylics 'always lift', and has a sore split beside the index nail. Identify the immediate contraindication, the lifestyle issue, the questions you would ask and a realistic alternative plan. Record your answer before discussing it with the group.***

Chapter 6 — Acrylic chemistry, the mix ratio and set time

Why product control protects the client and the professional

Monomer, polymer and what happens when they meet

A monomer is a small reactive molecule. A polymer is a long chain formed when many monomers join together. The liquid in an acrylic system is largely monomer; the powder is largely pre-formed polymer beads, with an initiator built into them.

When the brush carries liquid into the powder, the monomer begins to dissolve the surface of the polymer beads and releases the initiator. A catalyst in the liquid speeds the reaction. Monomer molecules join into chains around and between the existing beads, and the mixture changes from a workable bead into a solid mass. This is free-radical polymerisation.

The reaction gives off a small amount of heat. On a healthy nail with correct product control this is not usually noticed; if a client reports heat, stop, lift the hand away and check the ratio and the thickness being applied.

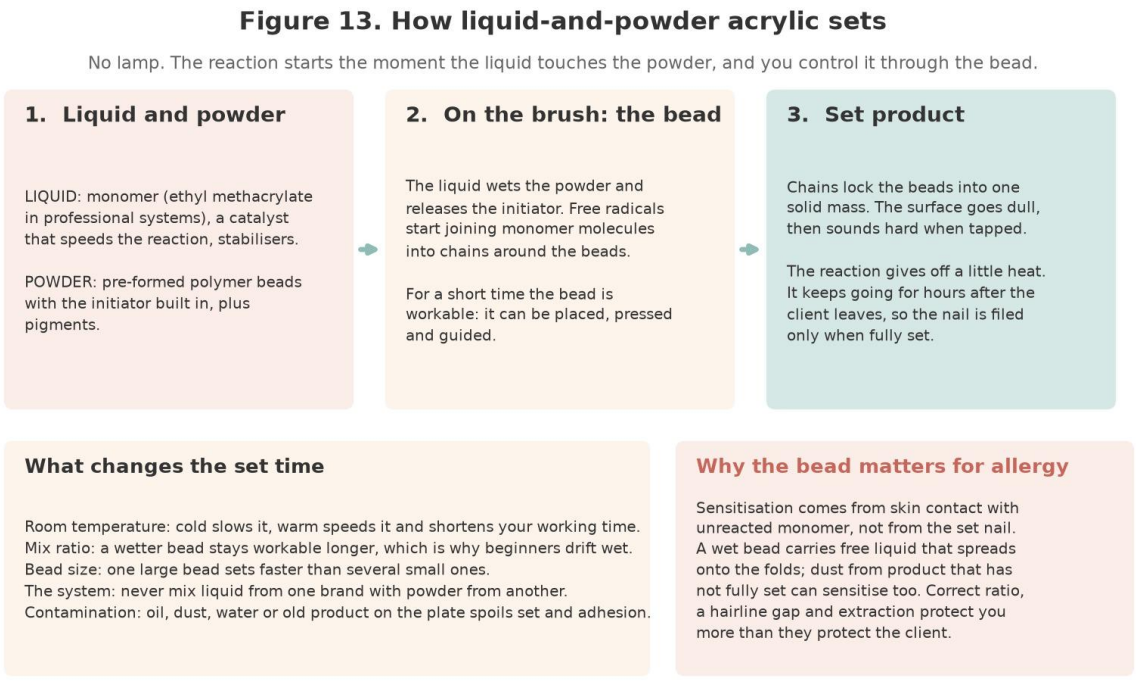


Figure 13. How liquid-and-powder acrylic sets. The liquid wets the powder and releases the initiator; chains form around the polymer beads and the workable bead becomes a solid mass without a lamp. Temperature, ratio, bead size and contamination change the set time; free monomer in a wet bead is the route to sensitisation.

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The mix ratio is the main control you have

The mix ratio is the balance of liquid to powder in the bead. It is judged by eye and by how the bead behaves, not measured. Learning to read a bead is the single most important practical skill in this course.

Bead	How it looks and behaves	What it does to the service
Too wet	Runs, spreads on its own, floods the cuticle area and sidewalls, stays shiny and soft for a long time	Weak, porous structure. Uncured monomer sits against skin — the main route to allergy. Service breakdown and lifting.
Correct	Holds its shape, settles gently when pressed, has a soft sheen and can be guided into place before it sets	Even, strong structure with good adhesion and predictable set time.
Too dry	Crumbly, dull, drags on the nail, will not smooth out, shows white specks or crystallisation	Poor adhesion, porous and brittle product, visible surface defects, lifting at the edges.

THE RATIO RULE — Wet beads are the most common cause of both allergy and lifting. If in doubt, wipe the brush on the lip of the dappen dish before picking up powder.

Figure 14. Reading the bead: too wet, correct, too dry

The mix ratio is judged by eye and by how the bead behaves on the brush. This is the course's most important skill.

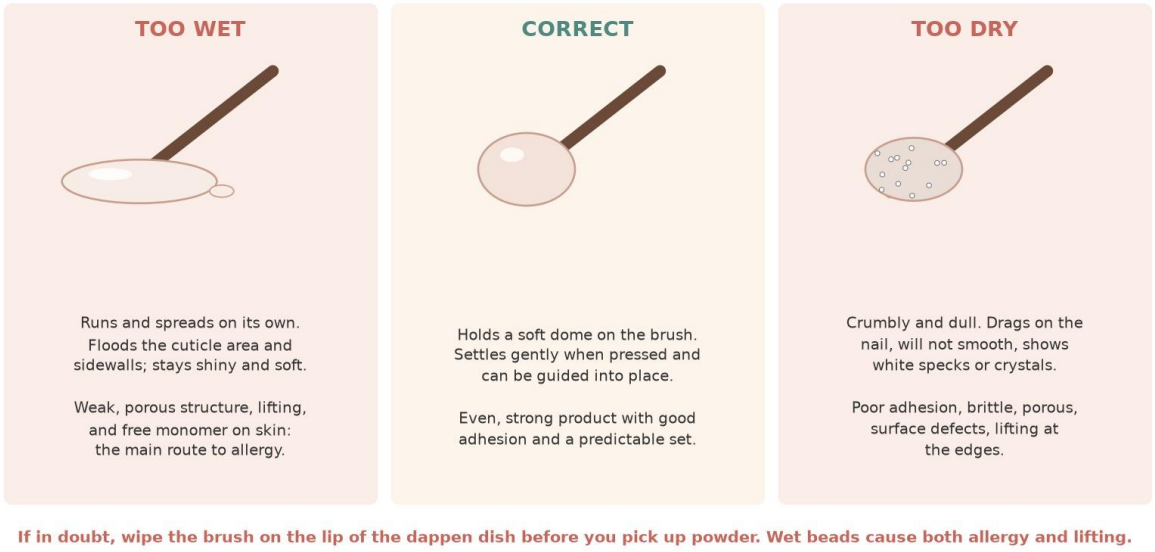


Figure 14. Reading the bead. Too wet runs and floods the folds; correct holds a soft dome and can be guided; too dry is crumbly and will not smooth. If in doubt, wipe the brush on the lip of the dappen dish before picking up powder.

Original illustration © The Looking Glass Beauty School, 2026.

Set time and what changes it

- Product chemistry: different systems are formulated to set at different speeds. Never mix liquid from one brand with powder from another.
- Room temperature: a cold room slows the set; a warm room speeds it and shortens your working time.
- Mix ratio: a wetter bead takes longer to set and stays workable, which is why beginners drift towards wet beads and build weak nails.
- Bead size: a large bead has more material reacting and sets faster than several small ones.
- Contamination: oil, dust, water or product residue on the plate interferes with both set and adhesion.

EMA and MMA — what a professional must know

The monomer used in professional systems is normally ethyl methacrylate (EMA). Methyl methacrylate (MMA) is a smaller, harder molecule that has been associated with severe natural nail damage, nail-plate separation and allergic reaction when used as a nail monomer.

The accurate legal position in Great Britain is this: methyl methacrylate is not listed as a prohibited substance in Annex II of the retained Cosmetic Products Regulation. However, every cosmetic product placed on the market must be safe for its intended use, and MMA nail liquid is widely regarded across the professional sector as unsuitable for application to the nail. It is prohibited for nail use in many jurisdictions outside the UK. Do not repeat the common claim that MMA is 'illegal in the UK'; explain the safety position instead.

Warning sign of an MMA-type product	What a professional should do
A strong, unusual or fruity chemical smell quite unlike your usual monomer	Stop using it. Obtain the Safety Data Sheet and check the monomer named.
Product that will not soak off in acetone in a reasonable time	Treat as a strong indicator. Do not attempt to drill it off the natural nail.
Enhancement so hard that it shatters rather than flexes, taking nail plate with it	Record the damage, remove as gently as possible and refer if the plate is compromised.
Product sold unusually cheaply, unlabelled, or without a Safety Data Sheet	Do not buy or use it. A supplier who cannot provide an SDS cannot support your COSHH duties.

YOUR SYSTEM — Record the exact monomer and powder you are trained on, and keep the Safety Data Sheet for each. Under COSHH you must base your assessment on the actual product, not on acrylic in general.

Why acrylate allergy happens

Sensitisation is caused by repeated skin contact with uncured product, not by the finished enhancement. Once a person becomes sensitised the reaction is usually permanent, and it can end a career as well as affecting dental and medical treatment later in life. Both the client and the professional are at risk; the professional is exposed far more often.

- Keep uncured product off the skin at all times — the client's and your own.
- Do not touch product with bare fingers to shape it.
- Work with a correct ratio; wet product spreads onto skin.
- Change gloves and wipe down; do not let the brush handle become coated in product.
- Control dust — filings of not-fully-reacted product can also sensitise.
- Never use a product as a 'patch test' on skin.

Chapter 7 — Nail architecture: apex, stress area, C-curve and balance

The part of the job that decides whether the nail lasts

Strength in an enhancement comes from its shape, not its thickness. A correctly shaped nail distributes force through the structure; a badly shaped one concentrates force at a single point and breaks there, however much product has been used.

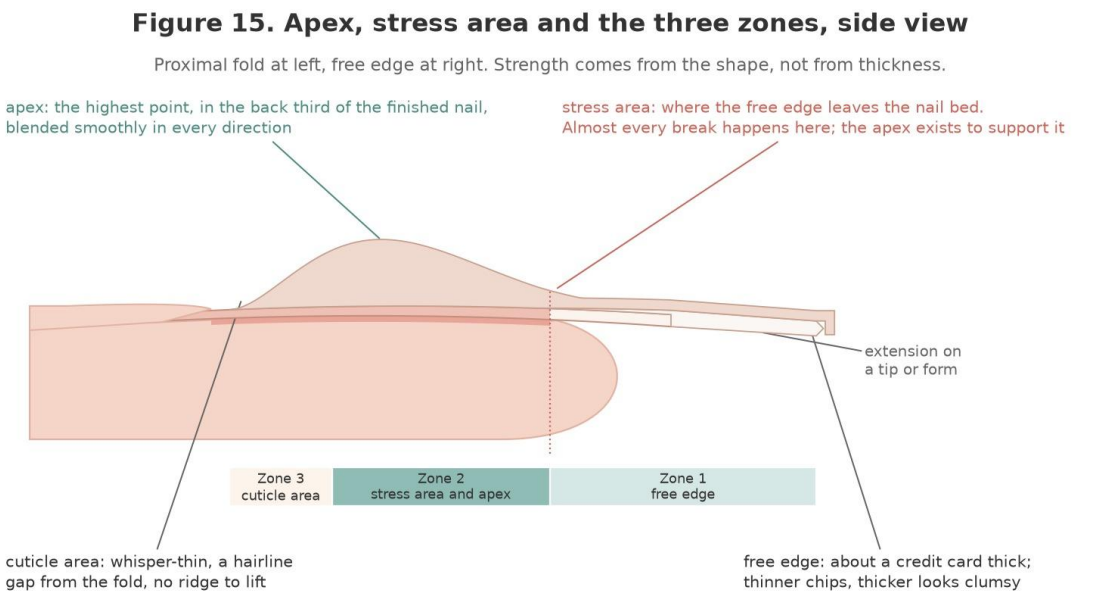


Figure 15. Apex, stress area and the three zones, side view. The apex sits in the back third of the finished nail and supports the stress area where the free edge leaves the bed; zone 3 is whisper-thin with a hairline gap, and the free edge is about a credit card thick.

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The four things a balanced nail needs

Feature	What good looks like	What goes wrong without it
Apex	The highest point of the enhancement, placed in the back third of the nail and blended smoothly in every direction	Apex too far forward: the nail snaps at the stress area. Apex too flat: the nail flexes and cracks. Apex too high: it looks bulbous and catches.
Stress area	The zone where the free edge leaves the nail bed, supported by the apex behind it	Unsupported stress area is where almost all breaks occur.
C-curve	An even transverse curve, consistent from sidewall to sidewall along the length	A flat or uneven C-curve makes the nail weak and the sidewalls look pinched or splayed.
Sidewalls	Straight when viewed from above, meeting the natural sidewall without a step or gap	Product over the sidewall causes lifting and skin irritation; a gap traps moisture and debris.

Thickness

Thickness should be greatest at the apex and taper in every direction. At the cuticle area the product must be whisper-thin so there is no ridge to lift. At the free edge, a useful reference is the thickness of a credit card; thinner and it will chip, thicker and it looks and feels clumsy.

Working in zones

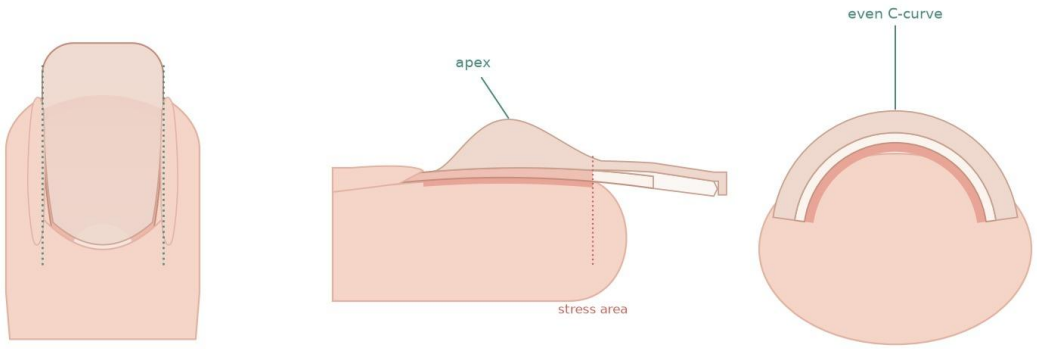
Most educators teach acrylic application in three zones. The zones are a way of controlling where product goes and how much, not a rigid rule about bead count.

Zone	Where	What it does
Zone 1	The free edge or the extension beyond the natural nail	Establishes length and shape. Usually placed first when working on a form.
Zone 2	The stress area and the centre of the nail	Builds the apex and carries the structural load. The most important bead on the nail.
Zone 3	The cuticle area	Blends the structure to nothing. Thin, neat and never touching living tissue.

CHECK FROM EVERY ANGLE — Look down the barrel of the nail from the free edge to see the C-curve, from the side to see the apex, and from above to see the sidewalls. A nail that looks right from only one angle is not balanced.

Figure 16. Check from every angle before the product sets hard

Top view for the sidewalls, side view for the apex, barrel view for the C-curve. A nail that looks right from one angle only is not balanced.



1. TOP VIEW
sidewalls straight and parallel, shape symmetrical, matched across all ten nails

2. SIDE VIEW
apex in the back third, blended with no shelf; thin at the cuticle, credit-card thin at the edge

3. BARREL VIEW
look down the nail from the free edge: an even C-curve, the same thickness sidewall to sidewall

Check all three on every nail while the product can still be guided. Once it has set, a fault can only be filed, and filing removes support.

Figure 16. Check from every angle while the product can still be guided: top view for the sidewalls, side view for the apex, barrel view for the C-curve.

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Nail shapes and their structural demands

Shape	Structural note
Square	Corners are vulnerable. Keep sidewalls straight and the free edge short enough to survive the client's work.
Squoval	Softened corners reduce snagging while keeping strength. A practical default.
Round and oval	Naturally strong shapes; force is distributed around the curve.
Almond	Needs a well-placed apex and even sidewall taper. Do not thin the sides to create the shape.
Coffin or ballerina	The narrow free edge removes support. Requires a strong apex and a conservative length.
Stiletto	The most demanding shape. Only appropriate for a client whose lifestyle allows it, and only with excellent architecture.

Figure 17. Nail shapes and their structural demands (top view)

The free edge is the pale part standing past the fingertip. Every shape needs a correctly placed apex; the narrow and long ones need the best architecture.



Figure 17. Nail shapes and their structural demands. Every millimetre of length adds leverage on the natural nail; the narrow and long shapes need the best architecture and a client whose lifestyle allows them.

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Chapter 8 — Kit, brushes, forms, tips, files and product storage

Select the right tool, use it lightly and maintain it correctly

Core application kit

Item	Purpose and safe use
Acrylic brush	Usually kolinsky or synthetic, sizes 8 to 12 for general work. The brush shapes the bead; it is not a scrubbing tool. Never stand a brush on its bristles or clean it in acetone.
Dappen dish	Holds a small working amount of monomer. Use a lidded glass or purpose-made dish,

	keep it covered when not in use to limit vapour, and never return used liquid to the bottle.
Polymer powder	Keep pots closed. Decant a working amount rather than dipping repeatedly into a large pot. Contaminated powder ruins the set.
Liquid monomer	Store upright, capped, away from heat and direct sunlight. Keep the Safety Data Sheet accessible.
Nail dehydrator	Removes surface moisture immediately before product. Apply to the plate only.
Primer	Acid-free primers are standard for most systems. Acid primers are corrosive to skin and should only be used where the manufacturer requires it, with appropriate control measures.
Tips and tip adhesive	Fitted sidewall to sidewall. Adhesive is a cosmetic product with its own hazards; keep away from skin and eyes.
Sculpting forms	Fitted under the free edge with no gap. Single use.
Files and buffers	See the grit table below. Single-client use or fully decontaminated between clients.
Lint-free wipes	Single use, one nail at a time. Never move contamination from nail to nail.
Dust brush	Soft, cleaned regularly. Brushing dust into the air is not dust control.
Local extraction	A source-capture extractor at the working point, not a desk fan. See Chapter 9.

Files and grits

A lower number is a coarser file. Coarse files are for product only. The natural nail plate is never shaped with a coarse file.

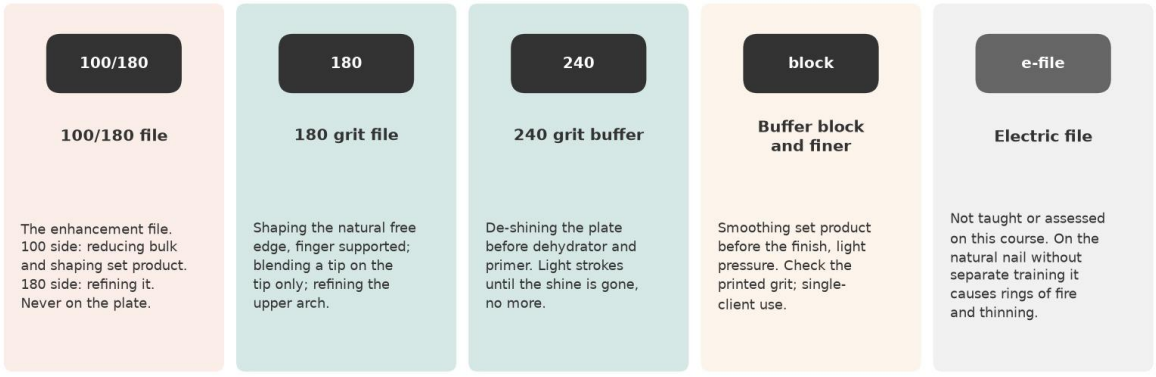
Grit	Category	Typical use
80–100	Very coarse	Reducing bulk on cured product only. Never on the natural nail.
100/180 combination	Coarse / medium	The standard enhancement file: 100 side for shaping product, 180 side for refining. This is the file used on this course.
180–240	Medium to fine	Refining the surface and shaping the free edge.
240+ and buffers	Fine to very fine	Smoothing before finish.

		Light pressure only.
Sponge buffer block	Varies	Conforms to the surface. Check the printed grit. Single-client use.

ELECTRIC FILES — An electric file is not taught or assessed on this course. Using one on the natural nail without separate, specific training causes rings of fire, plate thinning and burns. If your school offers e-file training, it is a separate activity with its own assessment.

Figure 18. Files and buffers for acrylic work

A lower number is coarser. Coarse files are for product only; the natural nail plate meets nothing coarser than 240 grit.



Keep the file flat on the surface; digging with a corner carves a flat spot. Files that cannot be decontaminated are single-client items.

Figure 18. Files and buffers for acrylic work. The 100/180 is for set product only; the natural nail plate meets nothing coarser than 240 grit, and the electric file is outside this course.

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Storage, decontamination and disposal

- Store monomer and powder according to the Safety Data Sheet: cool, dark, upright, away from ignition sources.
- Keep products in their original labelled containers. Decanting into unlabelled pots breaches COSHH and hides the hazard information.
- Clean brushes with monomer only, reshape and store flat or bristle-up.
- Decontaminate reusable metal tools by cleaning then disinfecting or sterilising according to the risk assessment. Files that cannot be effectively decontaminated are single-client items.
- Dispose of monomer-soaked wipes in a lidded metal or fire-retardant bin, emptied daily. Do not leave saturated wipes open on the desk.
- Check expiry dates. Old monomer sets unpredictably.

Chapter 9 — Health, safety, hygiene and ergonomics

Preventing exposure, cross-contamination and avoidable injury

Risk assessment and hierarchy of controls

The safest approach removes or reduces a hazard before relying on personal protective equipment. A salon COSHH/risk assessment should consider the product, task, route of exposure, frequency, quantity, ventilation, people at risk and emergency action.

Control level	Acrylic example
Eliminate	Do not stock a recalled/non-compliant product; do not perform an unsafe service.
Substitute	Select a professional compliant system with clear safety information where this reduces risk, while recognising that 'HEMA-free' is not allergy-free.
Engineering control	Use suitable local exhaust/downdraught extraction for dust and adequate general ventilation.
Administrative control	Training, consultation, thin application, closed containers, cleaning schedules, limited exposure time and incident reporting.
PPE	Task-appropriate gloves, eye protection and apron where the assessment/SDS requires them. PPE is the last line, not the only control.

Routes of exposure

Route	Examples	Control
Skin	Monomer, wet product, primer, dehydrator, acetone, filing dust, contaminated gloves	No skin contact; controlled application; prompt washing; suitable gloves; hand care
Eyes	Splash, dust, contaminated fingers	No touching face; eye protection for splash/dust tasks; eyewash access; follow SDS first aid
Inhalation	Fine filing dust, vapours and aerosols	Avoid unnecessary dust; local extraction; ventilation; closed containers; suitable RPE only where risk assessment requires
Ingestion	Eating/drinking at workstation, contaminated hands	No food/drink/smoking in treatment area; wash hands before breaks
Injection/cut	Nipper/file injury or	Protect skin, use tools

	contaminated sharp	correctly, stop bleeding service and follow blood-spill procedure

Cleaning, disinfection, sanitation and sterilisation

These terms are not interchangeable. Cleaning physically removes dirt and organic matter and must occur before effective disinfection or sterilisation. Sanitation reduces contamination to an acceptable routine level. Disinfection inactivates many disease-causing microorganisms but may not destroy bacterial spores. Sterilisation is a validated process intended to destroy all viable microorganisms, including spores.

Item/method	Correct approach
Hands/skin	Wash with liquid soap and water when visibly dirty and at the start/end of work; dry thoroughly. Use an appropriate hand rub on visibly clean hands where policy allows. Moisturise between services/after work to protect the barrier.
Work surface, extractor and dappen dish	Clean visible soil, then apply a compatible disinfectant for the stated contact time. Do not spray liquid into the extractor's electrical parts.
Porous files, buffers, orangewood sticks	Treat as single-client/single-use unless the manufacturer and local policy provide a validated decontamination route. Do not share between clients.
Metal pusher, nippers, clippers	After every client: safely transport as used, clean with detergent/mechanical action, rinse/dry, inspect, then disinfect using a compatible product at correct dilution/contact time. Sterilise by validated steam autoclave if the risk assessment/local authority requires it.
Blood-contaminated reusable tool	Stop, wear appropriate PPE, quarantine safely and follow the blood/body-fluid procedure. Dispose as appropriate or use a validated cleaning and steam-sterilisation route; do not simply wipe and reuse.
Autoclave	Steam steriliser used only for compatible instruments with validated cycles, packaging/loading, records, routine tests and maintenance. It is the recognised salon method when sterilisation is necessary.
UV cabinet/light box	Storage aid for already clean/decontaminated instruments. It must not be described as a reliable steriliser.

Glass-bead unit, hot-air oven, boiling water	Do not present as validated salon sterilisation. Current local-authority guidance commonly states that glass-bead units, UV boxes, hot-air ovens and boilers must not be used for sterilisation.
Chemical immersion	Usually disinfection in salon use. A product may make a sterilant claim only under specific validated conditions; follow label, COSHH and local rules and do not casually call a disinfectant 'sterilising solution'.

Standard between-client sequence

1. Remove used disposables and place them directly into the correct waste container.
2. Move reusable tools to a designated closed 'used' container; keep them separate from clean tools.
3. Clean and disinfect the workstation, hand rest, extractor touch points, dappen dish and any reusable containers using the correct contact time.
4. Perform the full tool-cleaning and disinfection/sterilisation process. Dry, inspect and store protected.
5. Wash and dry hands, replace couch roll/towel and set out only the clean items needed for the next client.
6. Record completed scheduled cleaning and any contamination incident.

Blood or skin-break procedure

1. Stop the service and move products/tools away.
2. Ask the client to apply pressure with clean disposable gauze where appropriate; the professional should wear suitable disposable gloves.
3. Do not continue product application over broken skin or active bleeding.
4. Dispose of contaminated single-use materials in the route identified by the local waste assessment. Quarantine reusable tools for validated processing.
5. Clean and disinfect the contaminated surface using the salon's blood-spill procedure and product instructions.
6. Wash hands after glove removal, document the incident and provide appropriate first-aid/referral advice.

Ventilation, dust and odour

HSE guidance for nail bars and beauticians (HSE COSHH Essentials sheet SR13, "Nail bars and other beauty services") identifies dermatitis, skin allergy, asthma and respiratory symptoms as relevant risks from beauty products and artificial-nail dust. General ventilation helps, but opening a window alone may not control dust at the technician's breathing zone. Use source capture where the assessment requires it, maintain filters and use methods that create the least dust.

- Keep bottles closed when not dispensing and clean spills immediately according to SDS.
- Position local extraction close enough to capture dust before it crosses the breathing zone.
- Do not rely on a strong smell to judge danger; some hazardous exposure has little odour, and smell perception varies.
- Do not use an ordinary desk fan that blows dust and vapour across the client or room.
- Empty/maintain extraction and waste in accordance with instructions while wearing any identified PPE.

Ergonomics and prevention of backache/RSI

Area	Good practice
Chair and feet	Use an adjustable supportive chair. Sit back with feet flat or on a footrest; avoid perching or twisting.
Desk and elbows	Use a narrow, stable manicure desk so client and technician do not reach. Support both forearms; aim for relaxed shoulders and elbows near 90 degrees.
Neck and vision	Bring the client's hand into the lit working area instead of bending the neck toward the desk. Consider task lighting or magnification.
Wrist and grip	Keep wrist as neutral as possible, use a light tool grip, support the finger and vary movement. Replace worn files rather than pressing harder.
Client position	Seat the client squarely facing the desk with supported arm and relaxed hand. Ask them not to pull, twist or hold the fingers rigid.
Work pattern	Insert microbreaks, stretch hands/forearms/neck, alternate tasks, schedule realistic service gaps and review symptoms early.
Warning signs	Persistent pain, tingling, numbness, weakness or night symptoms require workload review and appropriate medical/occupational-health advice.

30-SECOND POSTURE RESET — Feet supported; hips back; shoulders down; client close; forearms supported; wrist neutral; extraction correctly positioned. Reset between every client.

Dust and vapour — the two exposures specific to acrylic

Acrylic work produces two exposures that a gel-polish service does not: fine dust from filing cured product, and monomer vapour from the open dappen dish and the working bead. Both are controlled under COSHH, and both are the professional's own health as much as the client's.

Dust

- Filing dust from enhancements is fine enough to be breathed deep into the lungs, and it can contain product that has not fully reacted.
- Use source capture — an extractor at the point of filing that draws dust away from both faces. A desk fan moves dust around; it does not remove it.
- A paper or fabric face covering is not respiratory protection. Where the risk assessment requires it, use a properly fitted FFP2 or FFP3 respirator.
- Wipe surfaces with a damp cloth rather than brushing dust into the air. Vacuum with a suitable filter; do not sweep.
- Clean the extractor filter on the manufacturer's schedule and record it. A blocked filter is not a control measure.

Vapour

- Keep the dappen dish lidded whenever you are not drawing from it.
- Decant only the amount you need for the client in front of you.
- Ventilate the room: general ventilation plus source capture at the workstation.
- Close product bottles immediately after use.
- Dispose of monomer-soaked wipes in a lidded bin, emptied at the end of every day.

> ***YOUR OWN EXPOSURE — A client is exposed for two hours a fortnight. You are exposed every working day. Sensitisation ends nail careers; the controls in this chapter exist mainly to protect you.***

Figure 19. Dust and vapour: the controls at the workstation

Acrylic work produces two exposures gel polish does not. Both are COSHH exposures, and both are mainly about your own health.

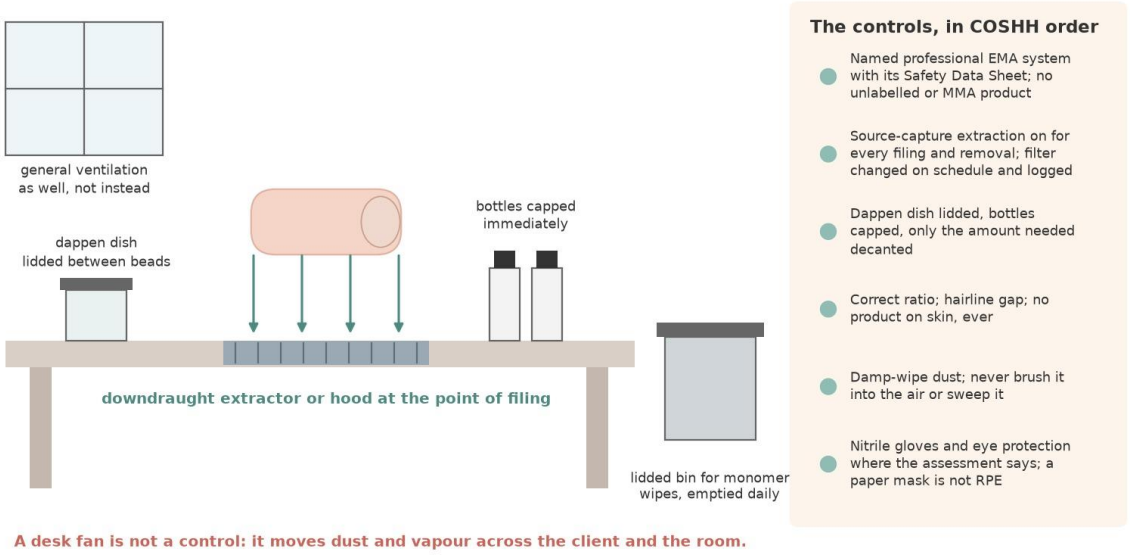


Figure 19. Dust and vapour controls at the workstation: source-capture extraction at the point of filing, the dappen dish lidded between beads, bottles capped, a lidded bin for monomer wipes and general ventilation as well. A desk fan is not a control.

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Chapter 10 — UK legislation, insurance, data and advertising

How the rules connect to everyday salon decisions

Legislation changes and local requirements differ. The following summary explains relevance to acrylic enhancement work in Great Britain as at October 2026; it is educational guidance, not legal advice. Businesses must check current official sources, insurer terms and their own local authority before delivery and at each document review.

Core legislation and guidance

Law or requirement	Relevance to an acrylic professional
Health and Safety at Work etc. Act 1974	Creates general duties to protect employees and others, including clients, so far as reasonably practicable. In practice: risk assessments, safe systems, training, maintained equipment, consultation and incident action.
Control of Substances Hazardous to Health Regulations 2002 (COSHH)	Requires employers to assess and prevent or adequately control exposure to hazardous substances. Keep SDS, identify skin/inhalation/eye/fire risks, use control measures, train staff, manage spills and review assessments when products or

	processes change.
Personal Protective Equipment at Work Regulations 1992, as amended in 2022	Where risk remains after other controls, employers must provide suitable PPE free of charge, train workers and maintain it. The 2022 amendment extends relevant duties to limb (b) workers. PPE selection must follow the hazard and SDS, not salon fashion.
Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013 (RIDDOR)	Specified work-related injuries, dangerous occurrences and diagnosed occupational diseases must be reported by the responsible person. Not every chip, rash or complaint is RIDDOR-reportable. Work-related diagnosed dermatitis can be reportable where the work involves significant/regular exposure to skin irritants or sensitisers. Use HSE's current decision guidance.
Data Protection Act 2018 and UK GDPR	Client contact details, consultation records, health information and photographs are personal data; health data is special-category data. Identify an Article 6 lawful basis and an Article 9 condition, give privacy information, minimise collection, secure records, set justified retention periods, support rights and separate treatment consent from marketing consent.
Cosmetic Products Enforcement Regulations 2013 and retained Cosmetic Regulation 1223/2009, as amended	Cosmetic nail products supplied in GB must meet safety, responsible-person, labelling and notification requirements. Buy through reputable compliant supply chains, keep original labels/batches, act on recalls and do not manufacture, relabel or import products without understanding responsible-person obligations.
Cosmetic Products Regulation (Restriction of Chemical Substances) (Amendment and Transitional Provisions) Regulations 2026 (SI 2026/23, as supplemented by SI 2026/109) — TPO	TPO is a photoinitiator used in light-cured gel products, not in liquid-and-powder acrylic; the entry is here because most acrylic professionals also stock gel polish or a gel top coat. GB restrictions on TPO begin on 15 August 2026 with transitional provisions for products already placed on the market; existing compliant stock may continue to be made available to 14 February 2027. TPO has been banned in the EU and Northern Ireland since 1 September 2025. Check current OPSS/Trading Standards and supplier guidance before purchase or use.
Local Government (Miscellaneous Provisions)	In England outside London, local authorities

Act 1982	can adopt licensing/registration provisions for specified treatments and may impose local byelaws or special-treatment conditions. Coverage differs between authorities, so each business and learner must check the authority where they practise and meet insurer requirements.
London Local Authorities Act 1991	In London, premises offering nail treatments generally need a Special Treatment Licence, applied borough by borough with local conditions on qualifications, hygiene and premises. Check the relevant borough before trading.
Civic Government (Scotland) Act 1982	In Scotland, licensing of relevant treatments and premises operates under this Act through the local licensing authority. Check the authority where you practise.
Public Health (Wales) Act 2017	In Wales, this Act established a special-procedures licensing framework. Nail services are not currently within the mandatory licensing categories, but practitioners in Wales must check current Welsh Government and local-authority guidance.
Electricity at Work Regulations 1989	Electrical systems and work equipment must be maintained to prevent danger and used safely. Visually check the extractor, any lamp used for a gel top coat, leads and plugs; keep liquids away; remove faults from use and arrange competent inspection/testing based on risk. The law does not automatically demand annual PAT testing for every appliance.
Waste Electrical and Electronic Equipment Regulations 2013 (as amended)	Extractors, lamps and other electrical equipment must enter an appropriate WEEE reuse/recycling route when discarded. Do not place electrical items in normal waste; use supplier take-back or an authorised business-waste route and retain records where required.
Consumer Rights Act 2015	Services must be performed with reasonable care and skill, information relied on by the consumer can become binding, and pricing must be clear. Avoid unrealistic guarantees and maintain complaint/rectification procedures.
Employers' Liability (Compulsory Insurance) Act 1969	Most businesses employing staff must hold employers' liability insurance meeting legal

	requirements and display/make the certificate accessible. This is separate from public and treatment liability.
Workplace (Health, Safety and Welfare) Regulations 1992	Relevant to safe, clean and suitable workplaces, including ventilation, lighting, temperature, welfare and workstation conditions.
Equality Act 2010	Service providers and educators must not unlawfully discriminate and should consider reasonable adjustments. Adjustments can change access or assessment method but cannot remove essential safety and competence standards.

Whatever the nation, always confirm licensing and registration requirements with your own local authority and your insurer before trading.

Public liability, treatment risk and product liability

Public liability insurance is not one of the Acts listed above, but it is an essential business control and may be required by venues, landlords, local authorities, training providers or contracts. A nail professional should confirm that cover includes the exact treatments, products, teaching activities, students/models, premises and working arrangements. Treatment/professional liability and product liability may be included or separate. Policy conditions often set qualification, age, consultation, record-retention, patch-testing and incident-notification requirements.

- Never assume the salon policy covers a self-employed professional or educator; obtain written confirmation.
- Notify the insurer promptly about an adverse reaction, injury, complaint or potential claim according to the policy. Do not admit liability or alter records.
- Keep factual, contemporaneous treatment and training records for the period agreed with the insurer and documented in the business retention schedule.
- If employing anyone, confirm employers' liability and health-and-safety duties in addition to public/treatment cover.

Data protection in consultation and case studies

Principle	**Salon action**
Transparency	Give a privacy notice explaining controller identity, purposes, lawful basis/condition, recipients, retention, security and client rights.
Data minimisation	Ask only what is necessary for safe treatment. A full GP address or unrelated medical history is normally unnecessary.

Health data	Document the relevant Article 6 lawful basis and Article 9 condition. Explicit consent may be available where genuinely appropriate, but treatment consent and data-processing conditions are different legal concepts.
Security	Lock paper files; use access-controlled, backed-up systems; avoid client details in personal messaging/photo galleries; restrict staff access.
Accuracy	Confirm changes at every appointment and record what the client said without turning observations into a diagnosis.
Retention	Use a written schedule based on purpose, insurer/legal advice and limitation periods; review and securely delete/anonymise when no longer needed.
Photographs	Use separate optional consent. State educational/assessment/marketing uses individually, avoid identifying features where possible and never post a case-study image automatically.
Breach response	Contain, assess, document and report qualifying personal-data breaches to the ICO within required timescales where applicable; inform affected people when high risk requires it.

Advertising Standards Authority (ASA) and CAP guidance

Advertising includes websites, booking pages, social media, paid posts, influencer content, leaflets and claims made in before/after images. The ASA/CAP position is that beauty marketers must hold evidence for objective efficacy claims and must not exaggerate effects.

Avoid	**Use instead**
'Guaranteed not to lift for three weeks'	'Designed to last between infills; individual wear varies with nail condition, lifestyle and aftercare.'
'Repairs weak or damaged nails'	'Adds strength and length over the natural nail. It does not diagnose or treat the cause of nail weakness.'
'Completely safe', 'non-toxic' or 'allergy-free'	Describe specific product information accurately and acknowledge that adverse reactions are possible.
'Treats fungus', 'kills infection' or 'cures green nail'	State that suspected infection cannot be treated cosmetically and requires medical/pharmacy advice.
'Qualified nail technician' based only on this	'Completed the Liquid and Powder Acrylic

CPD activity	Enhancement CPD Course' alongside the learner's genuine underlying qualification.
Heavily retouched before/after images or inconsistent lighting	Use comparable lighting/angle, retain originals, obtain consent and disclose relevant editing/filters.
'Best', 'number one' or award claims without evidence	State the exact verifiable award, year and category; do not imply a wider win than was achieved.

ASA RESOURCE — Learners should review the current ASA/CAP 'Beauty and Cosmetics: General' guidance before advertising: https://www.asa.org.uk/advice-online/beauty-and-cosmetics-general.html

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Electrical and extraction checklist

- ☐ The extractor, and any lamp used for a gel top coat, is obtained from a reputable supplier and is suitable for the task.
- ☐ Plug, cable, adaptor, housing and timer show no damage, heat marks or exposed parts.
- ☐ The extractor filter is clean and changed on the manufacturer's schedule, and the change is logged.
- ☐ The equipment is kept away from liquids and connected without unsafe adaptors or trailing-trip hazards.
- ☐ User checks, inspection/test intervals and maintenance are based on risk and recorded; an arbitrary annual sticker is not treated as the only control.
- ☐ Faulty equipment is unplugged, labelled and removed from use pending competent repair or WEEE disposal.

Chapter 11 — Preparing the natural nail for an enhancement

Adhesion is won or lost here

Almost all lifting is a preparation failure, not a product failure. The plate must be clean, dry, free of oil and free of loose non-living tissue, with no product on the living skin.

Preparation sequence

1. Sanitise your own hands and the client's hands. Inspect every nail and the surrounding skin before you touch a product.
2. Remove any existing product or polish. Do not begin preparation over old product.
3. Shorten and shape the free edge to the agreed length using a 180 grit file, supporting the finger and filing in one direction where the plate is fragile.
4. Gently push back the proximal nail fold with a rubber-tipped or wooden tool. Do not cut living tissue and do not force the tool under the fold.

5. Remove only loose, non-living cuticle tissue adhering to the plate, using the least abrasive method that works.
6. Remove the surface shine from the plate with a fine buffer (240 grit or finer), using light strokes. This is de-shining, not thinning. Do not etch grooves and do not use a coarse file.
7. Remove dust with a clean brush, then cleanse the plate with the product specified by the system.
8. Apply dehydrator to the plate only, and allow it to flash off.
9. Apply primer sparingly to the plate only, following the manufacturer's instruction. More primer does not mean more adhesion.
10. Work on one hand at a time so the prepared plate does not sit exposed and re-oil before product goes on.

> ***NEVER FILE THE PLATE TO CREATE GRIP — Thinning the natural nail does not improve adhesion; it weakens the plate and makes the client dependent on the enhancement. Correct cleansing and dehydration do the work.***

Figure 20. Preparing the natural nail for an enhancement

Almost all lifting is a preparation failure. De-shine, do not thin. Living tissue is never cut.



Figure 20. Preparing the natural nail for an enhancement: the free edge shaped, the fold pushed back, only loose non-living cuticle lifted, the plate de-shined with 240 grit, dehydrator and primer on the plate only, and a hairline gap kept at every fold.

Original illustration © The Looking Glass Beauty School, 2026.

Common preparation faults

Fault	Consequence
Cuticle tissue left on the plate	Product adheres to the tissue, not the nail; lifting at the base within days.
Oil or hand cream on the plate	Immediate adhesion failure.

Over-buffed, thinned plate	Sensitivity, weakness and a client who cannot go without product.
Primer or product on the living skin	Irritation, and a direct route to sensitisation.
Preparing all ten nails before starting product	The first nails re-oil before you reach them.

Chapter 12 — Tips and sculpting forms

Two ways to add length, both demanding an exact fit

Tips

1. Select a tip that matches the width of the natural nail from sidewall to sidewall. If you are between sizes, take the larger and file the sidewalls down — never pinch a narrow tip on.
2. Check the C-curve of the tip against the natural nail. A mismatched curve will lift.
3. Apply a small amount of adhesive to the well of the tip, not the nail plate.
4. Place at a shallow angle from the free edge, roll it down into position and hold until set, keeping adhesive off the skin.
5. Blend the tip so the join cannot be felt, using a 180 grit file or buffer on the tip only. Filing through the join into the natural nail is the most common tip fault.
6. Cut and shape to the agreed length before applying acrylic, holding the tip cutter at right angles to avoid shattering.

Sculpting forms

1. Choose the form shape that suits the nail and the finished shape you have agreed.
2. Fit the form under the free edge so it meets the hyponychium with no gap, and does not pinch the sidewalls.
3. Check the angle from the side: the form should continue the line of the natural nail, not point up or drop away.
4. Check from underneath that the form is symmetrical before applying any product.
5. Apply product, allow it to set fully, then remove the form by unpeeling rather than pulling.
6. Refine the underside and the free edge, checking the C-curve from the barrel view.

	Tips	**Forms**
Speed	Generally faster to learn	Slower to learn, faster once fluent
Cost per client	Consumable tip plus adhesive	Consumable form only
C-curve control	Limited by the tip you select	Fully controlled by the professional
Common fault	Filing through the blend into the natural nail	Gap under the free edge letting product touch the hyponychium

NO GAPS — Whether tip or form, a gap under the free edge lets product touch living tissue and traps moisture. Check underneath before you pick up the brush.

Figure 21. Tip and form: both demand an exact fit

Top: a tip fitted sidewall to sidewall (top view). Bottom: a sculpting form fitted under the free edge (side view).

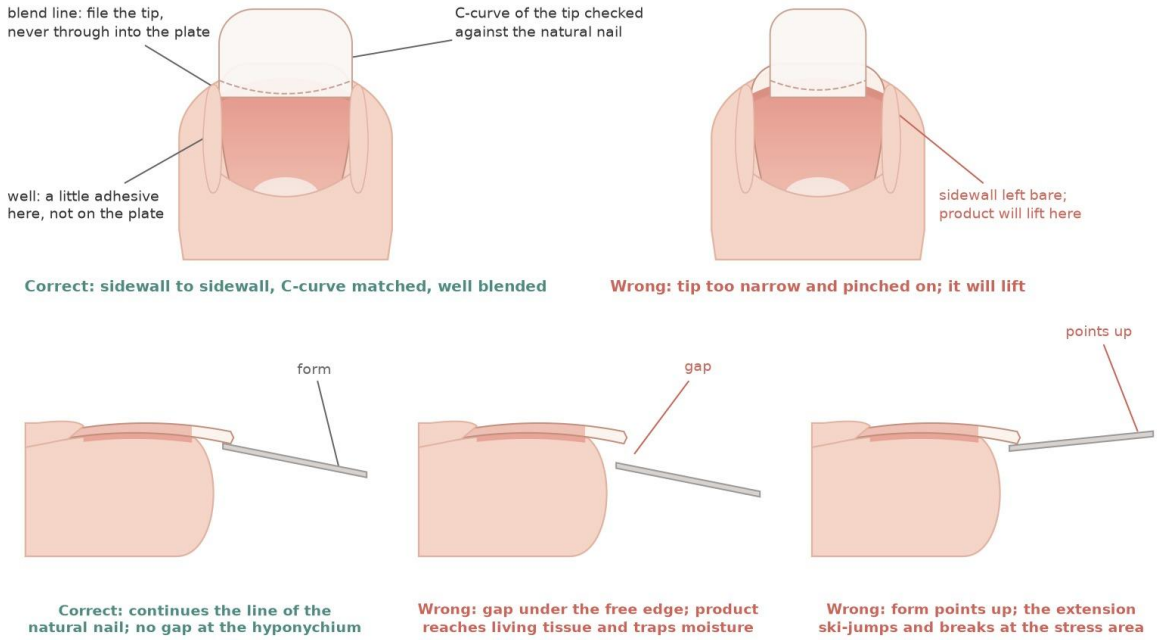


Figure 21. Tip and form. A tip is fitted sidewall to sidewall with its C-curve matched and the well, not the plate, carrying the adhesive; a form continues the line of the natural nail with no gap at the hyponychium and does not point up.

Original illustration © The Looking Glass Beauty School, 2026.

Chapter 13 — Acrylic application: bead control and building the structure

Placing product with intent

Setting up

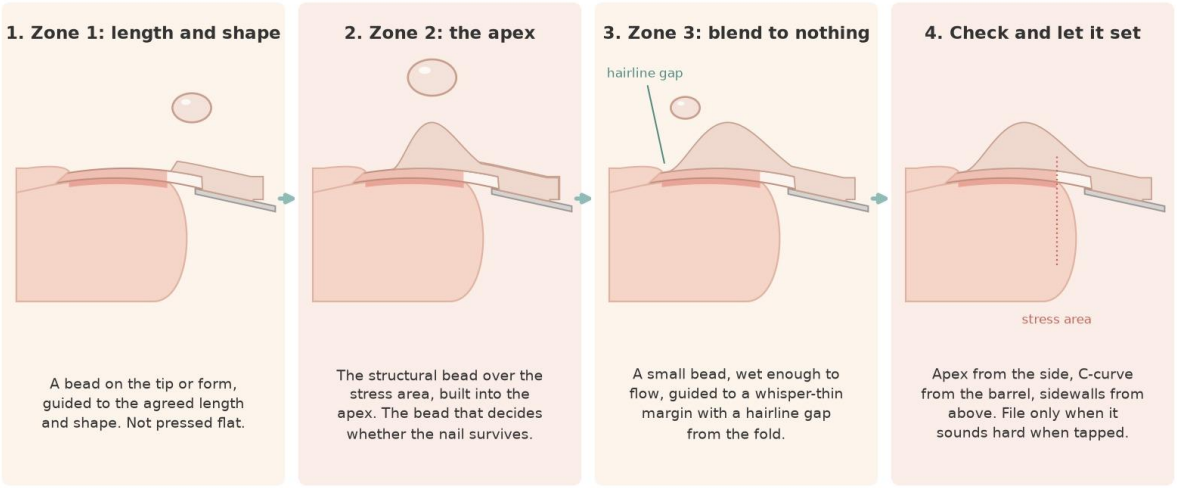
- Decant fresh monomer into the dappen dish and keep it lidded between beads.
- Saturate the brush, then wipe one side on the lip of the dish to remove excess.
- Pick up the bead by touching the powder surface and holding briefly; do not stir or dig.
- Pause for a moment before placing: the bead should settle into a soft, domed shape on the brush.

Application sequence

1. Place the first bead at the free edge or on the form, and guide it to the shape and length agreed. Do not press it flat.
2. Place the structural bead over the stress area to build the apex. This is the bead that determines whether the nail survives.
3. Place a small, wet-enough-to-flow bead at the cuticle area and guide it to a whisper-thin margin, leaving a hairline gap from the living tissue.
4. Blend the zones with the belly of the brush using light, flowing strokes. Do not paint back and forth once the product has begun to set.
5. Check the apex from the side and the C-curve from the barrel before the product sets hard.
6. Allow the product to set fully — it should sound hard when tapped — before any filing.

Figure 22. Building the structure: three beads, three zones

Side view at each stage. Place, guide, blend with the belly of the brush; do not paint back and forth once the product starts to set.



Zones control where product goes and how much; they are not a rule about bead count. Many professionals use more, smaller beads.

Product on the fold, the sidewall or the hyponychium is removed with the brush now, not filed off later.

Figure 22. Building the structure in three zones: a bead for length and shape in zone 1, the structural bead over the stress area in zone 2, a small wet bead blended to nothing in zone 3 with a hairline gap, then the three checks before the product sets hard.

Original illustration © The Looking Glass Beauty School, 2026.

Reading the surface as you work

What you see	What it means	What to do
The bead spreads on its own and floods the sidewalls	Ratio too wet	Wipe more liquid from the brush before picking up the next bead.
The bead will not smooth and shows crumbs	Ratio too dry	Carry slightly more liquid; check the powder is not contaminated.
Product creeps onto the	Bead placed too close, or too	Remove it now with the

cuticle area	wet	brush. Do not rely on filing it off later.
The surface sets before you have shaped it	Room too warm, bead too large, or working too slowly	Use smaller beads and commit to the placement.
White or cloudy patches appear in the set product	Poor ratio or contamination	Remove and rebuild that section rather than covering it.

THE HAIRLINE GAP — Product must never touch the proximal nail fold, the lateral folds or the hyponychium. A deliberate hairline gap at the cuticle area is correct technique, not a flaw. It grows out invisibly; product on skin causes lifting and allergy.

Chapter 14 — Filing, refining and finishing

Shaping the structure you built

1. Reduce and shape the free edge and sidewalls with the 100 grit side, keeping the file flat against the surface rather than digging with the corner.
2. Establish the shape from above: straight sidewalls, symmetrical, matched across all ten nails.
3. Refine the upper arch with the 180 grit side, blending the apex so there is no shelf or flat spot.
4. Thin the cuticle area to a whisper, checking by touch as well as by eye.
5. Check the C-curve from the barrel view and even it out.
6. Refine the underside of any extension, removing product ledges.
7. Buff through the grits to the finish the system requires, then remove all dust.
8. Apply finishing oil or the product specified, and check every nail from four angles before the client leaves.

Finishing fault	**How to see it**	**Fix**
Flat spot where the apex should be	Side view under a light	Rebuild rather than file the surrounding area down.
Ledge at the cuticle area	Run a fingertip across the margin	Thin the margin; do not cover with more product.
Uneven sidewalls	Look straight down the nail	Re-file both walls, do not chase one side.
Over-thinned free edge	Hold to the light — it will glow	Rebuild. A translucent edge chips within days.
Dust left under the free edge	Barrel view	Brush and wipe before finishing.

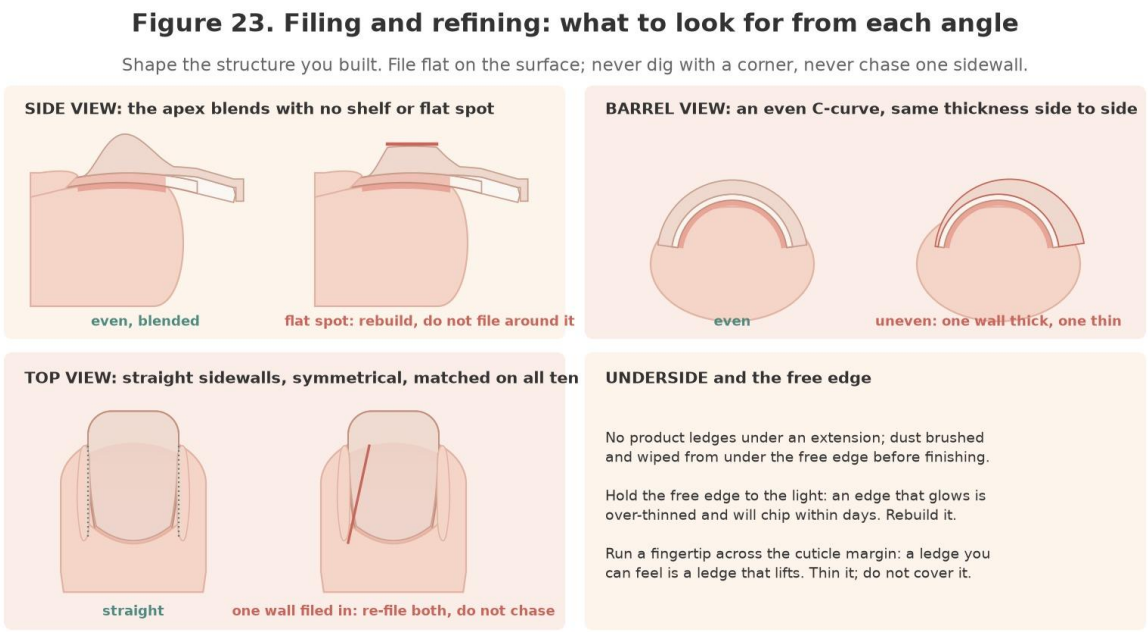


Figure 23. Filing and refining from each angle: the apex blended with no flat spot from the side, an even C-curve from the barrel, straight symmetrical sidewalls from above, and no ledges under the extension or at the cuticle margin.

Original illustration © The Looking Glass Beauty School, 2026.

Chapter 15 — Infills, rebalance, repairs and removal

Maintaining the structure, and taking it off safely

Infill and rebalance

An infill fills the regrowth area. A rebalance moves the apex back to where it belongs as the nail grows forward. Every maintenance appointment should do both; filling the gap without rebalancing is what causes nails to break at the stress area after a few cycles.

1. Assess every nail before touching a file. Record lifting, cracks, discolouration and any skin changes.
2. Remove any lifted product completely. Never apply fresh product over a lifted edge — that traps moisture and bacteria.
3. Reduce the bulk of the existing enhancement, lowering the old apex so the new one can be placed correctly.
4. Blend the regrowth area to a smooth transition with no ledge.
5. Prepare the exposed natural nail as in Chapter 11.
6. Apply product to rebuild the structure and reposition the apex.
7. Refine, finish and record what you did.

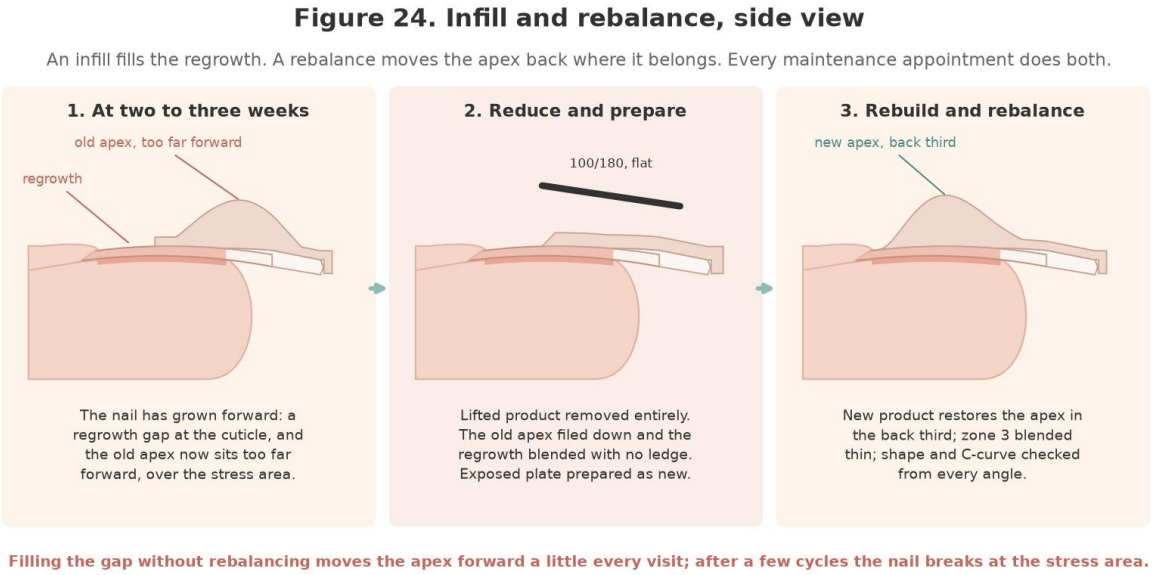


Figure 24. Infill and rebalance. At two to three weeks the old apex sits too far forward; lifted product is removed, the old apex reduced and the regrowth blended, then new product restores the apex in the back third.

Original illustration © The Looking Glass Beauty School, 2026.

Repairs

Presentation	Action
Small surface crack, nail otherwise sound	Reduce the area, rebuild with product, rebalance.
Crack through the stress area	Remove that nail's product back to sound material and rebuild. A patched structural crack will fail again.
Lifted corner	Remove the lifted product entirely, re-prepare and rebuild. Do not fill over it.
Nail broken with damage to the natural plate	Do not rebuild over damaged tissue. Remove, assess, and refer if the plate or bed is compromised.
Green or dark discolouration beneath the product	Stop. Do not cover. Follow the green-nail procedure in Chapter 4 and refer.

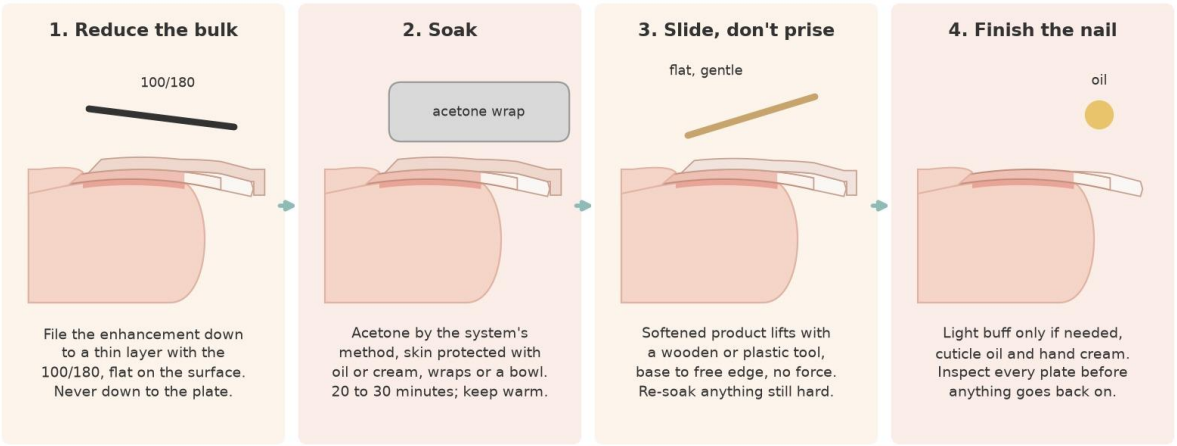
Removal

1. Explain the time removal will take and why it cannot be rushed.
2. Reduce the bulk of the enhancement with a file, leaving a thin layer. Do not file down to the natural nail.
3. Soak in acetone using the method the system specifies, protecting the surrounding skin.
4. Allow the product to soften fully — commonly 20 to 30 minutes. Re-apply acetone rather than scraping at product that is still hard.

5. Gently remove softened product with a wooden or plastic tool, working from the base towards the free edge with no force.
6. Repeat the soak for any remaining product rather than prising it.
7. Buff the plate lightly only if needed, rehydrate with oil, and advise on aftercare.

Figure 25. Removal in four stages

Removal is a service, not a chore. The damage clients blame on acrylic is almost always removal damage.



Never prise, nip, pick or drill an enhancement off. Levering it off takes layers of nail plate with it.

Product that will not soak off in a reasonable time is a warning sign of an MMA-type product: do not drill it off the natural nail.

Figure 25. Removal in four stages: reduce the bulk by file, soak in acetone for 20 to 30 minutes with the skin protected, slide the softened product off with no force, then buff lightly if needed and oil. Never prise, nip, pick or drill an enhancement off.

Original illustration © The Looking Glass Beauty School, 2026.

NEVER PRISE OR NIP — Levering an enhancement off takes layers of nail plate with it. The damage clients blame on acrylic is almost always damage caused by removal.

Chapter 16 — Longevity and troubleshooting

Reading a service that has gone wrong

Problem	Likely cause	Prevention
Lifting at the cuticle area	Cuticle tissue left on the plate, product touching skin, or product applied too thickly at the margin	Thorough preparation, hairline gap, whisper-thin margin.
Lifting at the free edge or sidewalls	Poor tip fit, gap under a form, or product not sealed to the edge	Check fit before product; cap the free edge.
Cracking across the stress area	Apex too far forward, or structure too thin	Rebalance at every infill; build the apex in the back third.

Snapping at the sidewall	Sidewall filed thin to create shape	Shape with the structure, not by removing support.
Yellowing or discolouration of the product	UV exposure, smoking, some cleaning products, or old monomer	Manage client expectations; rotate stock.
Green discolouration under the product	Moisture trapped under a lifted area, associated with bacterial colonisation	Never fill over lifting. Follow the referral procedure.
Client reports itching, redness or swelling around the nails	Possible acrylate sensitisation	Stop the service, remove product safely, refer for medical assessment. Do not reapply.
Bubbles or porosity in the product	Ratio too wet, or the brush stirred air into the bead	Correct ratio; place beads, do not stir them.
Client reports heat during application	Thick application, or ratio issue	Stop, lift the hand away, apply thinner layers.

Setting expectations honestly

- Two to three weeks between infills is normal; it is not a sign that the service failed.
- Growth rate, work, hobbies and household chemicals all affect wear. So does whether the client picks.
- Enhancements do not 'make nails grow'. They protect the free edge so it is not broken off.
- The natural nail underneath is not damaged by acrylic correctly applied and correctly removed. It is damaged by over-filing and by forced removal.
- Never promise a length of wear. Explain what affects it and record what you advised.

Chapter 17 — Contra-actions, adverse reactions and emergencies

Recognise early signs, stop exposure and escalate appropriately

Contra-action or adverse reaction?

A contra-action is an unwanted response or service problem during or after treatment, such as heat discomfort, minor accidental skin abrasion, lifting or dryness. An adverse reaction is a harmful response to a product or process and may include irritant dermatitis, allergic contact dermatitis, respiratory symptoms, chemical eye exposure or burns. The professional does not diagnose the mechanism; they recognise signs, stop exposure, give proportionate first aid and refer.

Event	Possible signs	Immediate management
Warmth as product sets or during filing	Warmth or a burning feeling as a large or wet bead sets, or from heavy filing	Stop; lift the hand away; smaller beads, correct ratio, lighter filing; never tell the client to tolerate pain
Product on skin before it sets	Visible wet product, primer	Remove at once with the

	or monomer in the fold or on the skin	brush or a clean wipe before it sets; wash the skin; do not continue until the skin is clean
Minor skin break/bleeding	Nipper/file injury	Stop; first aid and blood procedure; do not apply product; document; refer if needed
Irritant contact reaction	Burning, redness, dryness or cracking after chemical contact	Stop exposure; remove wet product; wash with soap/water; follow SDS; advise medical/pharmacy help if persistent
Suspected allergic contact dermatitis	Itching, swelling, redness, blistering, fingertip changes; may affect eyelids/face/neck	Stop all relevant nail product exposure; do not re-test; advise GP/dermatology assessment; document product/batch/SDS
Eye exposure	Stinging, watering, redness or product splash	Follow SDS immediately; irrigate with clean lukewarm water for specified time, remove contact lenses if easy, obtain urgent medical advice
Respiratory symptoms	Cough, wheeze, chest tightness, dizziness or shortness of breath	Stop work, move to fresh air if safe, follow SDS/emergency plan. Call 999 for severe breathing difficulty or collapse
Severe allergic/emergency signs	Rapid facial/throat swelling, breathing difficulty, collapse, confusion	Call 999 immediately; follow emergency/first-aid training; do not leave the person alone
Post-service lifting, cracking or breakage	Enhancement separates, cracks or breaks	Assess promptly. Remove lifted product; inspect for moisture/discolouration; repair or rebuild only when the nail is healthy
Possible infection after service	Increasing pain, heat, swelling, pus, spreading redness or green discolouration	Do not treat or cover; advise urgent pharmacy/GP/NHS 111 assessment; emergency care if systemically unwell

General adverse-reaction response

1. **STOP** the treatment and prevent further exposure.

2. **REMOVE** uncured/wet product safely without spreading it; do not use additional sensitising solvents on skin unless the SDS directs.
3. **RINSE/WASH** or provide first aid according to the exact product SDS and first-aid training.
4. **ASSESS** urgency. Call 999 for emergency signs; otherwise advise NHS 111, GP, pharmacist or occupational health as appropriate.
5. **RECORD factual signs, times, affected area, product names/batches, the stage of the service, PPE and extraction in use, action, advice and witnesses. Photograph only with consent and without delaying care.**
6. **REPORT** internally, to insurer and under RIDDOR/OPSS rules where applicable. Preserve product/records; do not alter notes.
7. **REVIEW the COSHH assessment, product system, extraction, training and technique before any further service. Do not perform a salon 'patch test' to prove safety.**

Allergy prevention for professionals

- Keep uncured product off your own skin and the client's skin every time, not only when a reaction is suspected.
- Keep the dappen dish lidded, wipe the brush on the lip of the dish and never on a towel, clean bottle necks, and do not let the brush handle, your gloves or the desk become coated in product.
- Change contaminated gloves immediately and select glove material/change intervals from the SDS/COSHH assessment.
- Do not touch your face, phone, pen or door handles with contaminated gloves.
- Use source-capture extraction for every filing and removal; dust from product that has not fully set can sensitise as well as irritate.
- Report hand eczema, cracking, eyelid/facial rash, cough or wheeze early. Once sensitised, continued exposure can jeopardise a technician's career and future medical/dental choices.

Chapter 18 — Aftercare and professional client support

Clear advice protects the service and the natural nail

Aftercare advice

- Apply cuticle oil to intact skin daily and massage around, not beneath, the enhancement. Stop if irritation occurs.
- Wear suitable gloves for prolonged wet work, cleaning, gardening and chemical exposure; dry hands thoroughly afterwards.
- Do not pick, peel, bite or prise the enhancement. Do not use nails as tools to open cans, scrape labels or lift objects.
- Keep the chosen length practical. If an edge catches, contact the nail professional; do not tear it or glue a damaged natural nail without advice.
- Book an infill and rebalance every two to three weeks, and professional removal when you want to stop. Never leave a set to grow out or pull it off.

- Contact the salon promptly for lifting, a crack, trapped moisture, green/black/yellow change, unexpected white/powdery area or significant chip.
- Stop product exposure and seek advice for itching, redness, swelling, blistering, burning, fingertip changes or rash on hands/face/eyelids. Seek emergency help for breathing or throat/facial swelling.
- Do not cover a changed nail with ordinary polish or another enhancement before it has been assessed.
- Acetone removal may leave skin/nails temporarily dry; moisturise intact skin and keep nails short if not reapplying.
- Results vary: aftercare supports wear but cannot make an enhancement indestructible or guarantee a fixed number of days.

Written aftercare handout

CLIENT TAKEAWAY — LOOK AFTER YOUR ENHANCEMENTS: Oil intact cuticle skin daily. Wear gloves for wet/chemical work. Never pick or peel. Keep nails at a practical length. Book professional removal. Contact us for lifting, damage, colour change or discomfort. Stop exposure and seek medical advice for a suspected reaction; call 999 for breathing difficulty, collapse or rapid face/throat swelling.

Professional follow-up

- Record the advice given and the recommended review/removal date.
- Invite the client to report early problems without blame. Early lifting can be removed before moisture is trapped.
- At the next visit, compare wear with the previous plan and adjust length, shape, preparation or system only for a clear reason.
- Do not repeatedly repair the same failure without investigating product compatibility, equipment, technique, nail condition and lifestyle.
- Document complaints factually, preserve photographs/product records and follow the salon's complaint and insurer-notification procedure.

TEACH-BACK ACTIVITY In pairs, give the client aftercare in 60 seconds. The listener checks that you covered professional removal, wet-work protection, no picking, early reporting of lifting or colour change, and urgent reaction signs. Swap roles and improve one phrase.

Chapter 19 — Course conclusion and next steps

What you can now do, and where to go from here

What you have covered

Chapter 1 set out ten learning outcomes. By the end of the taught day, and once your three case studies are complete, you should be able to:

- say what liquid-and-powder acrylic is for, who it suits, and when gel polish, builder gel or no enhancement at all is the honest answer (LO1);
- name the structures of the nail unit and recognise the conditions that mean adapt, postpone or refuse (LO2);
- consult, record and gain informed consent, and write a treatment plan another professional could follow (LO3);
- explain monomer, polymer, initiator and catalyst, the mix ratio and set time, and the difference between EMA and MMA (LO4);
- select, handle, store and dispose of products and equipment under COSHH and the manufacturer's instructions (LO5);
- control dust and vapour at the workstation and explain why acrylate overexposure causes sensitisation (LO6);
- prepare the natural nail without damage to the plate or living tissue (LO7);
- fit and blend a tip, or fit a sculpting form, sidewall to sidewall with no gaps (LO8);
- build a balanced structure in zones with a controlled bead, a correctly placed apex, straight sidewalls and an even C-curve (LO9);
- refine, finish, infill, repair and remove an enhancement safely, and give aftercare (LO10).

Questions, feedback and reflection

Ask anything that is still unclear before you leave the taught day; a question asked now is a lifted nail avoided later. Complete the course evaluation form before you go: it is read, and the course changes because of it. The reflective practice form that goes with your case studies asks what went well, what you would change and what you need to practise; fill it in honestly, because it is for you.

Certificate, insurance and scope

- The CPD certificate is issued when the theory paper, the practical observation and all three case studies are satisfactory; attendance alone does not earn it.
- Before offering the service commercially, send your insurer the certificate and your underlying qualification and get written confirmation that liquid-and-powder enhancements are covered; check your local authority's licensing conditions, some of which prohibit MMA by name.
- Describe the training accurately: 'completed the Liquid and Powder Acrylic Enhancement CPD Course', never 'qualified' on the strength of a CPD day.

Building confidence

Start with overlays and short extensions on healthy natural nails, for clients who are not hard on their hands. Add length, sculpted forms and the demanding shapes once your beads are consistent, your apex lands in the back third without thinking about it, and your infills rebalance rather than fill. Keep a side and a barrel photograph of every set for the first three months and compare them; the improvement is quicker to see than to feel.

Further training and support

Acrylic is the strongest and most sculptable of the nail systems compared in Chapter 2. The natural next steps at The Looking Glass Beauty School are the Builder Gel Overlay and Short Extension CPD Course, for clients who want a lighter structural service, and the Gel Polish Nails CPD Course for colour over the natural nail and over enhancements; electric-file work and advanced sculpting are separate activities with their own assessment. Support continues after the course: contact the school by phone, message or email with questions about products, technique or a client you are unsure about. We would rather hear from you than have you guess. Details of every course and how to book are on the school's website and social media.

> ***THANK YOU — for choosing The Looking Glass Beauty School. Correct ratio, hairline gap, apex in the back third, extraction on, and nothing ever prised off: the rest is practice.***

Chapter 20 — Learner activities and knowledge checks

Work through these before, during and after the taught day

Before the taught day

1. Read Chapters 1 to 10 and note three questions to bring with you.
2. Obtain the Safety Data Sheet for the monomer and powder you will use. Write down the named monomer, the hazard statements and the storage requirements.
3. Write, in your own words, what a correct bead looks like and what a wet bead does to a client.
4. Sketch a side view of a nail and mark where the apex should sit.

Knowledge checks

#	Question
1	Explain in your own words what happens chemically when monomer meets polymer powder.
2	Give three things that change the set time of an acrylic bead.
3	Describe a correct bead, a wet bead and a dry bead, and the consequence of each.
4	Where does the apex belong and why does

	its position matter?
5	State the accurate legal position of MMA in Great Britain, and four warning signs of an MMA-type product.
6	Name the two exposures specific to acrylic work and one control measure for each.
7	Why is a hairline gap at the cuticle area correct technique?
8	Give four causes of lifting and the preparation step that prevents each.
9	Explain the difference between an infill and a rebalance.
10	A client's nail has green discolouration under a lifted corner. Set out exactly what you do, in order.
11	A learner says acrylic 'ruins your natural nails'. Answer them using what you now know about preparation and removal.
12	Why is acrylate sensitisation a greater risk to you than to your client?

Scenario activities

SCENARIO A — A regular client arrives with three nails lifted at the cuticle and one with a crack through the stress area. She has a wedding in four days. Set out your assessment, what you would do to each nail, what you would say about the crack, and what you would record.

SCENARIO B — Midway through a set you notice the beads are spreading and flooding the sidewalls, and the room is warmer than usual. Explain what is happening, what you change, and why leaving it would matter beyond appearance.

SCENARIO C — A new client asks for MMA product by name because it 'lasts longer and is cheaper'. Explain your response, the accurate legal position, and the safety reasoning.

After the taught day

- Complete three case studies as described in Chapter 1.
- For each, record the products used, the preparation carried out, the structure achieved, the aftercare given and one thing you would do differently.
- Photograph each set from above, from the side and from the barrel view, with consent, in the same light before and after.

Chapter 21 — Learner forms and client records

Templates to evidence consultation, planning, aftercare and case studies

DATA PROTECTION These forms collect personal and sometimes special-category health data. Do not ask for information that is not necessary for treatment safety.

Form A — Acrylic enhancement consultation and treatment plan

Provider data-protection details

Data controller (school/salon name)	_____
ICO registration reference	_____
Privacy notice location	_____
Lawful basis and special-category condition	_____
Retention period	_____

Template: the school or salon completes this block before the form is used with clients.

Client/model full name	_____
Date of birth / confirmation aged 18+	_____
Telephone or preferred contact	_____
Date and time	_____
Technician/learner	_____

Desired result and lifestyle

Desired colour/finish/shape/length	_____
Work and hand-use considerations	_____
Wet work/chemicals/sport/hobbies	_____
Previous enhancement services and typical wear	_____
Previous removal method and any damage	_____

Relevant health, allergy and nail questions

Relevant question	Yes/No	Necessary details
Have you ever had itching, redness, swelling, blistering, nail lifting or rash after gel, acrylic, nail glue, adhesive or dental material?		
Do you currently have pain, heat, swelling, broken skin, rash, discharge or colour change around any nail?		
Have you been diagnosed with a nail/skin condition that affects this treatment area? If yes, what treatment limitation has been advised?		
Are you taking medication or receiving treatment that has recently changed your skin, nails, healing or sensitivity? Record only relevant details.		
Do you have diabetes, impaired circulation, reduced sensation, immune suppression or another factor that requires extra care?		
Is there anything else relevant to your ability to sit for the service, remain positioned or follow aftercare?		

Professional nail and skin assessment

Area	Observation	Proceed/adapt/postpone/refer
Nail plates		
Nail folds/cuticle		
Free edges/hyponychium		
Colour/attachment		
Skin integrity		
Other		

Agreed treatment plan

System and named products	_____

Overlay, tip or form; length and shape agreed	_____
Preparation/adaptations	_____
Colour/finish/shape	_____
Removal/review date	_____
Price	_____
Aftercare supplied	_____

Treatment consent: I confirm that I have answered the relevant consultation questions honestly, had the planned acrylic enhancement service, foreseeable risks, alternatives, aftercare and costs explained, and had an opportunity to ask questions. I understand that results and wear vary and that I may ask the professional to stop at any time.

Client/model signature and date	_____
Technician/learner signature and date	_____
Tutor signature if assessed	_____

Data notice/condition: I have received or can access the provider's privacy notice. Where the provider relies on explicit consent to process relevant health data, that consent must be recorded separately and must meet ICO requirements.

Relevant data-processing record / explicit-consent statement if used

_____

Form B — Separate photograph consent

Photograph consent is optional and separate from treatment consent. Tick each use agreed. Refusing marketing use must not affect access to treatment.

- ☐ Assessment evidence viewed by The Looking Glass Beauty School educator/quality assurer.
- ☐ Accreditation evidence viewed by the accrediting body where required.
- ☐ Anonymous learner portfolio (no face/name/identifying jewellery).
- ☐ The Looking Glass Beauty School website/social media/marketing.
- ☐ The salon's website/social media/marketing.

How long images will be retained	_____
How consent can be withdrawn for future use	_____

Model name	_____
Model signature/date	_____
Learner/educator signature/date	_____

Form C — Treatment record

Record field	Details
Date / technician	
Consultation changes	
Nail condition before service	
Products, colour and batch/lot where practical	
Overlay, tip or form; length and shape	
Preparation and service steps	
Variations/contra-actions	
Outcome	
Aftercare and review date	
Client comments	
Signatures	_____

Form D — Three-case-study log

Case study 1

Model code/name	_____
Date	_____
Service and goal	_____
Nail/skin assessment	_____
Product system (monomer, powder, primer)	_____
Structure: overlay, tip or form; length and shape	_____
Preparation/adaptations	_____
Outcome and photograph filenames	_____
Aftercare	_____
What went well	_____
What I would improve	_____
Follow-up/wear result	_____
Educator feedback	_____
Result: Satisfactory / Further evidence required	_____

Case study 2

Model code/name	_____
Date	_____
Service and goal	_____
Nail/skin assessment	_____
Product system (monomer, powder, primer)	_____
Structure: overlay, tip or form; length and shape	_____
Preparation/adaptations	_____
Outcome and photograph filenames	_____
Aftercare	_____
What went well	_____
What I would improve	_____
Follow-up/wear result	_____
Educator feedback	_____
Result: Satisfactory / Further evidence required	_____

Case study 3

Model code/name	_____
Date	_____
Service and goal	_____
Nail/skin assessment	_____
Product system (monomer, powder, primer)	_____
Structure: overlay, tip or form; length and shape	_____
Preparation/adaptations	_____
Outcome and photograph filenames	_____
Aftercare	_____
What went well	_____
What I would improve	_____
Follow-up/wear result	_____
Educator feedback	_____
Result: Satisfactory / Further evidence required	_____

Form E — Adverse reaction / incident record

Client/learner/staff name	_____
Date/time/location	_____
Treatment stage	_____
Factual signs and symptoms	_____
Affected nail/skin/area	_____
Product names, batches and SDS	_____
Stage of the service and structure (overlay, tip, form)	_____
PPE and ventilation in use	_____
Immediate action/first aid	_____
Emergency service/NHS 111/GP/pharmacist advice	_____
Witnesses	_____
Photograph consent/file reference	_____
Insurer/internal/RIDDOR/OPSS notification decision	_____
Follow-up and risk-assessment review	_____
Completed by/signature/date	_____

Form F — Cleaning and equipment log

Date/time	Item/area	Clean/disinfect/sterilise	Product/cycle/contact time	Completed by	Fault/action

Form G — Product system record

This record is completed twice: first during training, using the system the tutor demonstrates, and again by the learner for their own kit before offering the service commercially.

Monomer (named)	Powder	Primer and dehydrator	SDS held (date)	Supplier and batch	Date checked

Form H — Learner declaration

I understand the scope of this CPD activity and will not describe it as a regulated qualification. I will work only within my underlying qualifications, competence, insurer and local requirements; use a named professional EMA system with its Safety Data Sheet; control dust and vapour; avoid

skin exposure; refuse suspected infection/reaction; and maintain accurate client and product records.

Learner name	_____
Signature	_____
Date	_____
Educator name/signature	_____

Chapter 22 — References and further reading

Authoritative sources used for technical and regulatory checking

Legislation and guidance can change; the provider reviews this list at least annually and whenever a product, insurer rule, local-authority condition or official safety alert changes.

1. UK Parliament — *Health and Safety at Work etc. Act 1974* — 1974 — https://www.legislation.gov.uk/ukpga/1974/37
2. UK Parliament — *Control of Substances Hazardous to Health Regulations 2002* — 2002 — https://www.legislation.gov.uk/uksi/2002/2677
3. Health and Safety Executive — COSHH Essentials sheet SR13: Nail bars and other beauty services (control approach 2, engineering control) — 2019 — https://www.hse.gov.uk/pubns/guidance/sr13.pdf
4. UK Parliament — *Personal Protective Equipment at Work Regulations 1992* (as amended by the Personal Protective Equipment at Work (Amendment) Regulations 2022) — 1992/2022 — https://www.legislation.gov.uk/uksi/1992/2966
5. Health and Safety Executive — *Personal protective equipment (PPE) at work guidance* — 2022 — https://www.hse.gov.uk/ppe/
6. UK Parliament — *Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013 (RIDDOR)* — 2013 — https://www.legislation.gov.uk/uksi/2013/1471
7. Health and Safety Executive — *RIDDOR: occupational diseases* — n.d. — https://www.hse.gov.uk/riddor/
8. UK Parliament — *Electricity at Work Regulations 1989* — 1989 — https://www.legislation.gov.uk/uksi/1989/635
9. Health and Safety Executive — *Portable appliance testing (PAT) FAQs* — n.d. — https://www.hse.gov.uk/electricity/faq-portable-appliance-testing.htm
10. UK Parliament — *Waste Electrical and Electronic Equipment Regulations 2013* — 2013 — https://www.legislation.gov.uk/uksi/2013/3113
11. UK Parliament — *Data Protection Act 2018* (with UK GDPR) — 2018 — https://www.legislation.gov.uk/ukpga/2018/12

12. Information Commissioner's Office — *Special category data: guidance* — n.d. — https://ico.org.uk/for-organisations/uk-gdpr-guidance-and-resources/lawful-basis/special-category-data/
13. Information Commissioner's Office — *How should we obtain, record and manage consent?* — n.d. — https://ico.org.uk/for-organisations/uk-gdpr-guidance-and-resources/lawful-basis/consent/
14. Information Commissioner's Office — *Data protection fee* — n.d. — https://ico.org.uk/for-organisations/data-protection-fee/
15. UK Parliament — *Consumer Rights Act 2015* — 2015 — https://www.legislation.gov.uk/ukpga/2015/15
16. UK Parliament — *Cosmetic Products Enforcement Regulations 2013* — 2013 — https://www.legislation.gov.uk/uksi/2013/1478
17. UK Parliament — *London Local Authorities Act 1991* (special treatment licences) — 1991 — https://www.legislation.gov.uk/ukla/1991/13
18. UK Parliament — *Local Government (Miscellaneous Provisions) Act 1982* — 1982 — https://www.legislation.gov.uk/ukpga/1982/30
19. UK Parliament — *Civic Government (Scotland) Act 1982* — 1982 — https://www.legislation.gov.uk/ukpga/1982/45
20. Welsh Parliament — *Public Health (Wales) Act 2017* (special procedures) — 2017 — https://www.legislation.gov.uk/anaw/2017/2
21. European Commission — *Commission Regulation (EU) 2020/1682* (HEMA and di-HEMA TMHDC restriction, professional use only; as retained in the GB Cosmetics Regulation) — 2020 — https://www.legislation.gov.uk/eur/2020/1682
22. European Parliament and Council — *Regulation (EC) No 1223/2009 on cosmetic products* (applies in Northern Ireland) — 2009 — https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=CELEX%3A02009R1223-20250901
23. UK Government — The Cosmetic Products Regulation (EC) No 1223/2009 (Restriction of Chemical Substances) (Amendment and Transitional Provisions) Regulations 2026, SI 2026/23 — TPO inserted as entry 1745 of Annex II; regulation 2(3) in force 15 August 2026, with transitional provision to 14 February 2027 — https://www.legislation.gov.uk/uksi/2026/23/made
24. UK Government — The Cosmetic Products Regulation (EC) No 1223/2009 (Restriction of Chemical Substances) (Amendment and Transitional Provisions) (No. 2) Regulations 2026, SI 2026/109 — further Annex II and Annex III amendments — https://www.legislation.gov.uk/uksi/2026/109/made
25. British Beauty Council — *What does the EU ban on TPO gel nail polish mean for manicurists?* — 2025 — https://britishbeautycouncil.com/what-does-the-eu-ban-on-tpo-gel-nail-polish-mean-for-manicurists/
26. British Association of Dermatologists — *Dermatologists issue warning about UK artificial nail allergy epidemic* — 2018 — https://www.bad.org.uk/dermatologists-issue-warning-about-uk-artificial-nail-allergy-epidemic

27. British Society for Cutaneous Allergy — *Acrylate allergy patient information* — n.d. — https://www.cutaneousallergy.org/
28. NHS — *Nail problems* — n.d. — https://www.nhs.uk/conditions/nail-problems/
29. NHS — *Fungal nail infection* — n.d. — https://www.nhs.uk/conditions/fungal-nail-infection/
30. Advertising Standards Authority / Committee of Advertising Practice — *Beauty and cosmetics: general advice online* — n.d. — https://www.asa.org.uk/advice-online/beauty-and-cosmetics-general.html
31. Cosmetic, Toiletry and Perfumery Association — *Guidance on nail products* — n.d. — https://www.ctpa.org.uk/
32. Skills England (formerly Institute for Apprenticeships and Technical Education) — *Nail services technician occupational standard* — n.d. — https://www.instituteforapprenticeships.org/apprenticeship-standards/nail-services-technician/
33. Braintree District Council — Update for Nail Technicians: Methyl Methacrylate (MMA) Monomer — 2012 — https://www.braintree.gov.uk/downloads/file/1063/updatefornailtechsmma
34. Dundee City Council — Public entertainment licence: nail bar conditions (the licence holder shall not use MMA liquid monomer) — n.d. — https://www.dundeecity.gov.uk/sites/default/files/public_entertainment_-_nail_bar_conditions.docx
35. European Chemicals Agency — Committee for Risk Assessment opinion on the harmonised classification of methyl methacrylate (adopted 18 March 2021: respiratory sensitiser category 1 proposed in addition to the existing skin sensitiser classification) — 2021 — https://echa.europa.eu/registry-of-clh-intentions-until-outcome
36. Health and Safety Executive — Research Report RR627: Health and safety in nail bars — 2008 — https://www.hse.gov.uk/research/rrhtm/rr627.htm
37. American Academy of Dermatology — Artificial nails: dermatologists' tips for reducing nail damage — n.d. — https://www.aad.org/nail-care-secrets/reduce-artificial-nail-damage

All references checked 27 October 2026 as part of the scheduled document review.